Women's Healthcare Associates LLC

# Clinical Workflows

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**Links to Additional Abortion Services Workflows:**

Surgical Abortions in IOS (GA 13w6d)

D&E Process (at Legacy Hospital for GA >14w or not a candidate for IOS GA >13w6d - Outside Referral

D&E Process (at Legacy Hospital for GA >14w or not a candidate for IOS GA >13w6d

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## Abnormal Breast Imaging Follow-up at WHA

Workflow Contact: Melanie Clausen

Teams Involved – Clinicians, Radiologist, Nurse Breast Health Navigators, Imaging Navigators

**Workflow:**

- Inconclusive mammogram is found by radiologist,
- Radiologist sends results to the results pool
- Imaging Navigator sends to Nurse Navigator's results folder in Epic.
- Nurse Navigators enter orders, call patient, notify them of results, and either refer them to another facility or get them scheduled for an appointment at WHA.
- This conversation is then routed to the providers so that they are aware that the patient is going to complete diagnostic imaging, but they don't need to take other action.
- After diagnostic imaging is complete, Imaging Navigators send results to Nurse Navigators and to the providers.
- If a biopsy is needed, Nurse Navigators enter orders for this.
- If there is any additional follow up needed, nurse navigators enter these orders and put 6-month follow-ups into a reminder system.

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# Abnormal Ovarian Finding

Workflow Contact: Amy Palmer, Kayla Quinn

Teams Involved – Clinicians, Advice RNs

**Ultrasound reader identifies abnormality of ovary and puts finding and recommendation in report.**

1. Responsible Provider receives report and documents plan in chart based on finding.

2. IT team prints a report monthly and sends to nurse managers.

3. Nurse Managers review and send the report to the advice nurses to initiate protocol.

The advice nurses use below smart phrase to ensure follow up occurs.

**ABNORMAL ULTRASOUND TRACKING**

**Situation: {bilateral/left/right:34462} {ovary/ies:34645} flagged as being abnormal on Abnormal Ultrasound report. Chart was reviewed and {AUS1:33245}**

- ○ Plan is clear & no guidance re: f/u is needed
- ○ Need guidance as to whether f/u is warranted.
- ○ Finding resolved with subsequent imaging. No further action needed.
- ○ Per clinican notes, no further f/u needed.

**Background:**

US performed on ***

Finding on report is ***

US reader made recommendations for f/u?: {YES/NO:25237}

Finding documented in notes by clinician: {YES/NO:25237}

Plan in place for follow up? {YES-EX/NO/NA:33175}

Documented in CPOE?: {YES/NO:25237}

Steps in place to ensure follow up occurs if f/u was recommended: {YES/NO:25237}

**Assessment**: {Assessment: 33247}

- ○ Plan is in place for follow up, and steps are in place to ensure f/u occurs
- ○ Plan is in place, but no steps in place to ensure recommended f/u occurs
- ○ No recommendations were made, need to consult pt's clinician to determine if flagged finding warrants any future follow up
- ○ Finding has resolved with subsequent US or surgery no further action needed
- ○ Per Clinician documentation no follow up is indicated.

**Recommendation**

{Recommendations: 33248}

- ○ Plan & steps are in place for f/u. No action needed.
- ○ Per Clinician no additional imaging needed at this time. No action needed.
- ○ Orders placed for f/u as was recommended by clinician in CPOE. Orders will automatically be followed up on in the scheduling orders work queue by front office
- ○ Plan was documented in OV or telephone encoutner, but problem not added to chart. Problem & provider's documented plan added.
- ○ Post dated order placed for f/u ultrasound, which will be automatically followed up on in the scheduling orders work queue by the front office.
- ○ Recommended F/U ultrasound or surgery already completed and issue has resolved. No further action needed for this finding.
- ○ Will consult clinician to determine if any surveillance imaging or action needed for this finding? RN to facilitate order entry and any necessary follow up that may be needed.

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# Cervical Cytology Result Management

Workflow Contact: Kayla Quinn, Amy Palmer

Teams Involved – Clinicians, Health Maintenance Navigators, Clinical Support Teams

**Clinician receives results from cervical cytology/HPV test, colpo or LEEP Pathology:**

- Normal results: result note sent to Pod Pool for MA/LPNs to notify patient

- Abnormal results (or pts who need sooner follow up than 3 or 5yrs):

  - Provider enters patient into pap tracker for Health Maintenance Navigator to recall patient for her future care.
  - Provider creates result note and sends to Pod Pool (or Advice pool if complicated results, or anticipates patient will have many questions. Generally speaking, the Pods are able to manage these results) for support staff to notify patient and coordinate any follow up care.
  - If provider notifies patient of abnormal pap results via myhealth, result note should also be sent to Pod Pool to ensure patient views her results and gets scheduled, and if patient has not viewed her results within 7 days, MA (or Advice depending on complexity of result) to attempt to reach patient by phone x3 to notify of abnormal results & need for follow up. If unable to reach a certified letter will be sent topatient.

**Resulting Cervical Cytology/Pathology – MA/LPNs/RNs:**

- MA/LPNs/RNs to always review the pathology report itself prior to notifying the patient to ensure matches up to provider's result note (safety net). Discuss with provider any discrepancies.

- While running weekly Pap Result reports, MA/LPNs to be verifying that patients were notified of their pap results and notified correctly, and that if pap was abnormal that the pap tracker was also completed. If the pap tracker was not completed for an abnormal result, a staff message should be sent to the provider for follow up.

**Ongoing Cervical Cytology Follow Up Workflow:**

Designated Individual (Health Maintenance Navigator), will continue to monitor the Pap Tracker Report for ongoing Pap, Colpo & LEEP follow ups.

When follow up identified to be needed, telephone encounter will be started and sent to the corresponding patient services team. Patient services team to reach out to patient to schedule with the following smart phrase included:

@FNAME@ is due for a {Pap/Colpo/LEEP:34162}. Called patient to see if she would like to schedule:

- If @FNAME@ is uncomfortable coming in to the office during COVID-19 pandemic for this follow up care, or has any questions as to why this needed, will offer to schedule her for a telehealth visit to discuss risks & benefits of postponing her follow up care.
- If @FNAME@ declines both,will forward to the provider for FYI.
- If unable to reach @FNAME@, after 3 phone attempts (1 call each week), will send a Myhealth message if applicable & then a certified letter as needed.

{lmsg / reached patient:38545}

@MECRED@

1. 3 attempts should be made to reach out to patient to schedule by phone (1 call per week).
2. If unable to reach by phone, and patient is active on Myhealth, send Myhealth message to patient & wait 1 week

   a. General outreach in Pap Tracker will be updated by individual running pap tracker report to reflect most recent contact and next date need to re-review or reach out to patient.

3. If no response, a certified letter should be sent & the telephone encounter should be CC'd to the provider for FYI and to advise if any further outreach needed.
   a. After certified letter receipt received, patient will be marked as N/A in the pap tracker. A report can then be run post COVID-19 pandemic to try to reach out to these patients again.

During COVID-19 Pandemic, any patients who wants to postpone, should be scheduled for a telehealth visit with the provider to discuss risks/benefits of postponing her follow up and discuss safe timing for follow up if postponing, and added to the Waitlist for in person visit.

If the patient declines both, forward telephone encounter to clinician for FYI.

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# Mifepristone for Early Pregnancy Loss

Workflow Contact: **NWPE:** Suzanne, Amber, Shelly **LD:** Caitlin, Adrienne, Chanelle **GW:** Chanelle, Carissa **South Hub:** Ashely, Kristina

Teams Involved – Clinicians, Clinical Services

***Candidates for Regimen:** Women with an ultrasound-documented intrauterine pregnancy loss at 9w6d or less.

**Exclusion Criteria:**

- Unable to document intrauterine pregnancy or concern for ectopic pregnancy
- Known severe anemia with HGB <9.5
- An IUD in place (Must be removed if patient desires to proceed.)
- Current long-term systemic corticosteroid therapy
- Chronic adrenal failure
- Known coagulopathy or anticoagulant therapy
- Intolerance or allergy to mifepristone or misoprostol
- Severe liver, renal or respiratory disease, uncontrolled hypertension, cardiovascular disease
- Patient unwilling or unable to adhere to care instructions, not available for follow-up contact*, cannot understand instructions due to language or comprehension barriers.

**Workflow Details:**

1. Patient chooses medical management of her early pregnancy loss.
   a. She has in-person appointment with a clinician to administer the mifepristone regimen.
   b. Clinician will utlilize smart set for diagnosis, orders, AVS smart phrase, and nursing follow up referral. (See attached tip sheet!)
2. **Lab work should be ordered and scheduled if it is needed.**
   a. Labs that should be ordered: ABO/Rh if unknown and ABC if concern for anemia
3. Once patient is deemed appropriate for Mifepristone, the medication may be administered by the clinician. Clinical staff can obtain medication from their site designated team member to hand-off to the clinician.

4. Provider will utilize smart set to initiate orders and the follow up protocol for early pregnancy loss.

**Medication Regimen:**
Mifepristone 200mg PO x 1 dispensed by WHA and taking it is witnessed by the provider in the clinic.
Misoprostol 800mcg prescription sent to the pharmacy for patientto take 24hrs after mifepristone bucally or vaginally. (can be taken up to 72 hours later)
Rhogam is ordered and given, if necessary, when mifepristone is given.
Anti-emetics and pain medication are chosen from the smart set and sent to the pharmacy.

- PATIENT MUST SIGN DANCO "PATIENT AGREEMENT FORM" FOR MIFEPREX even though it's for an EPL. Please just cross out termination on the form for the patient.
- The patient is sent home with an information packet and a copy of the agreement form given by the support staff.
- Here is the link to the "Patient Agreement forms" and education materials to review. https://www.earlyoptionpill.com/for-patients/download-mifeprex-materials/

**Follow-up**: See below.
Acceptable ways to confirm pregnancy has expelled:

a. Patient's report of bleeding. When patient and clinician both feel expulsion has happened based on symptomatology, they are correct 96-99% of the time.
b. Transvaginal ultrasound demonstrating GS is absent
c. HCG drop of >/=80% in one week
d. Negative UPT 4 weeks after AB
e. Contraception (if applicable): IUD can be placed one week after SAB/MAB. All other methods can be started as soon as the day of misoprostol administration.

5. Recommended Follow up during COVID Crisis:
   a. Provider will use smart set to initiate recommended follow up protocol.

**Workflow Details for Clinical Services:**

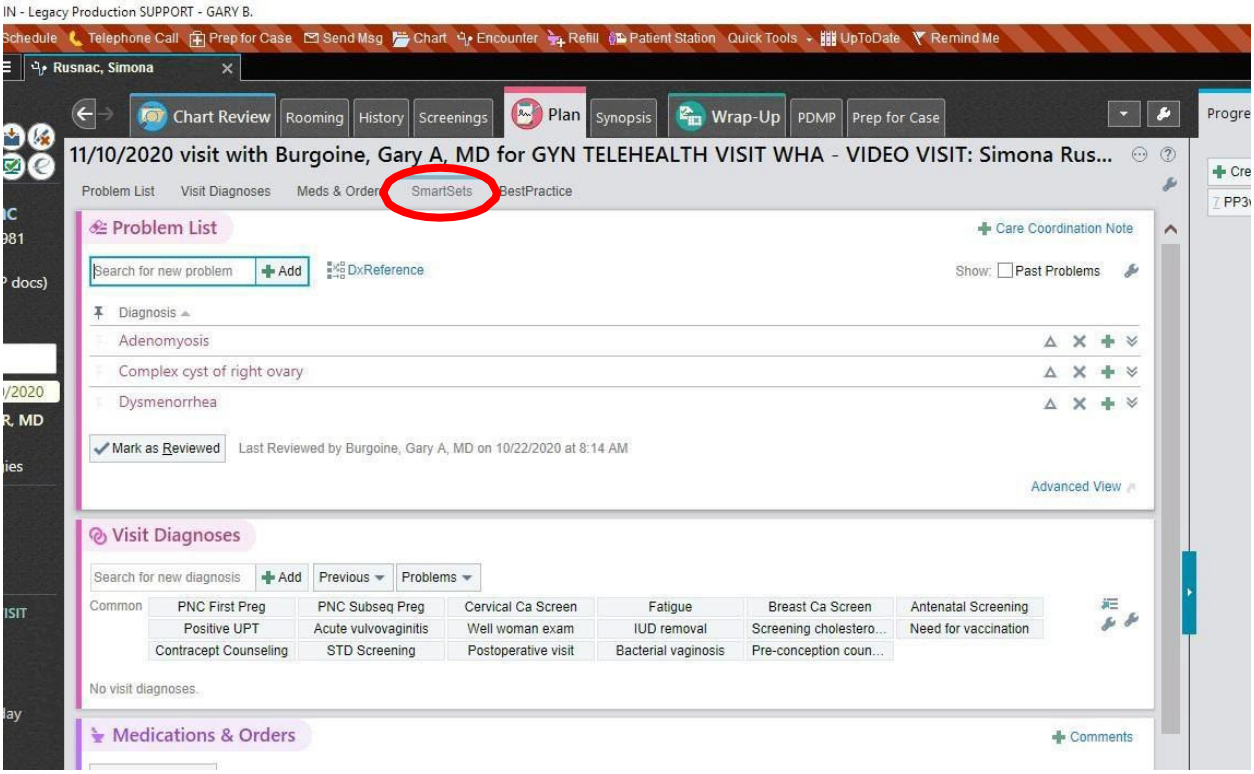
1. Room patient in standard fashion.
    a. Chief Complaint – "Other" In comments add, early pregnancy loss.
    b. Progress note – Pull in GYN problem visit.
        i. Provider must utilize Smart Set to pull in Diagnosis, orders, charting template, nursing referral, and AVS.
            1. Patient will take Mifeprex in office during visit.
2. Consent Form should have patient label placed in upper right hand corner. Once patient has signed the consent form, a copy should be given to the patient and the original should be sent to scanning.
3. Patient should be sent home with Mifeprex medication guide, patient brochure. https://www.earlyoptionpill.com/for-patients/download-mifeprex-materials/
4. MRN added to log sheet to track and reconcile Mifeprex counts.

**Workflow Details for Advice:**

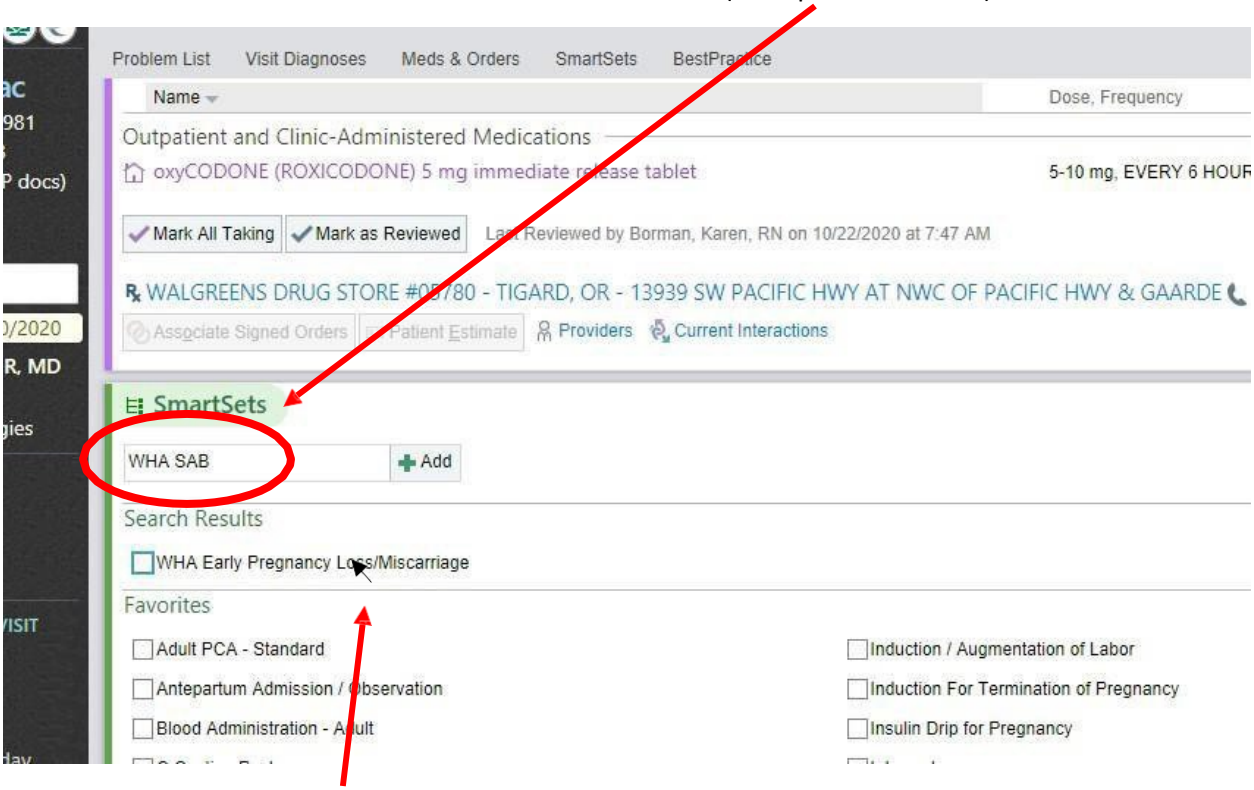
Utilizing the smart set correctly will initiate a nursing referral order for follow up. These are managed by the RNs out of their advice referral work queue.

## How to find and favorite a SmartSet

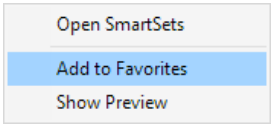
1. From Plan activity, go to SmartSets:




2. Enter terms or name into “Search for new SmartSet” (example: “WHA SAB”):




3. RIGHT click the SmartSet label “WHA Early Pregnancy Loss/Miscarriage” and you’ll get the option to Favorite it.




Now the WHA Early Pregnancy Loss Smart Set will always be in SmartSets foryou!

**External Imaging**

***(Providence and other facilities outside of WHA and Legacy)***

Workflow Contact: Caitlin MacKenzie

Teams Involved – Referral Navigators, Clinical Team, Clinicians

Clinician enters orders as outlined below:

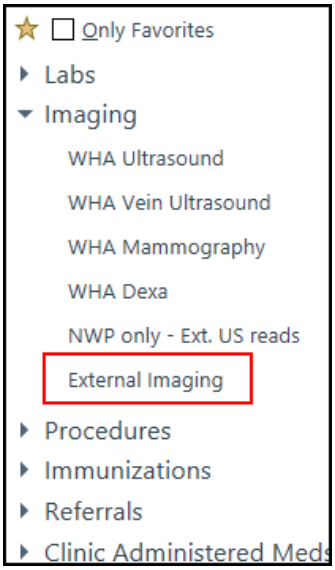
1. Click the **Preference List** button 

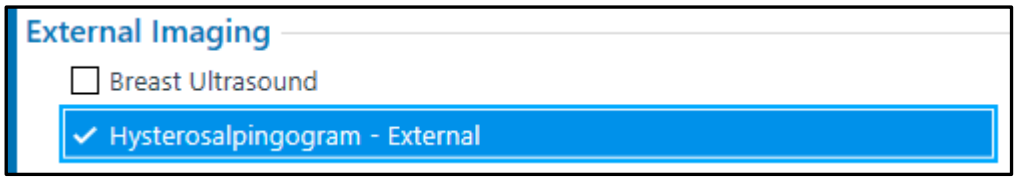

   ***Note**: It's important to use the **Preference List***
   

   *as*

   *this will give the order the correct shell, meaning it will function in the way that it needs to in orderfor the Referral Navigators to track the information that isnecessary.*

2. Under the **Imaging** category click **External Imaging**

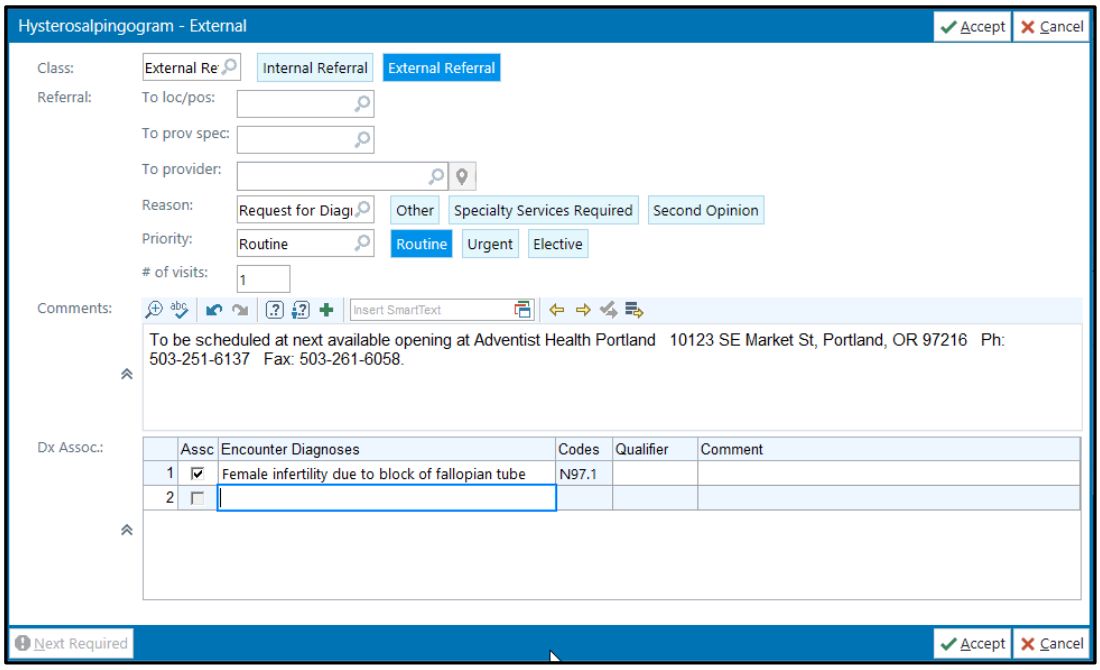





3. Click the order needed

4. Complete the smartphrase in the **Comments** box and associate a diagnosis code, then click **Accept.**

   **Note**: Clinical staff does not have to fill out any of the information in the **Referral** section of the order (The **To loc/pos**, **To provider** boxes) due to the information already residing in the **Comments** box.

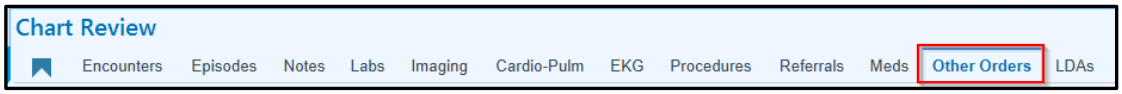



5. Sign the order 


Clinician's work is complete.

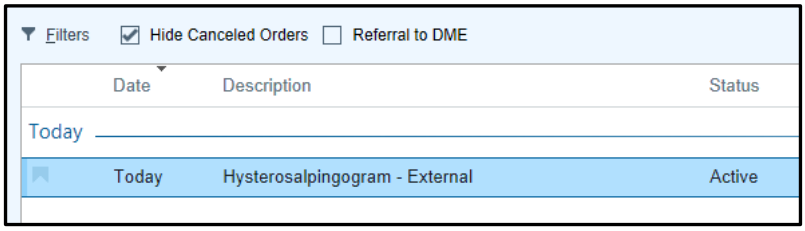
6. Go to **Chart Review** 

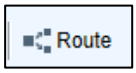
7. Go to the **Other Orders** tab



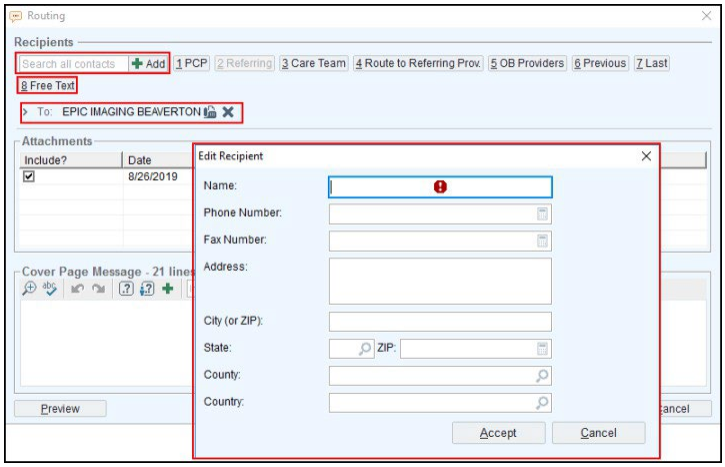

***Note**: Routing the order from this tab gives the facility you're routing it to all the order information they need without getting a wet signature from the patient's WHA provider. This needs to be done AFTER clinician cosign if support team entered theorder.*

8. Select the order(s)




9. Click the **Route** button 

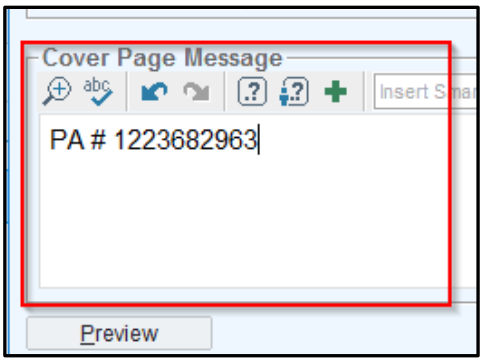

10. Use the search box to find the location the order is being sent to **OR** select **Free Text** and type the information into that box then click **Accept**




11. Click **Send**

***Note:*** *For advanced imaging orders that require Prior Authorizations, the PA number must be included on the Cover*

*Page once the PA has been approved.*




12. The referral navigator will then look at the referral shell and make sure it was faxed.
    If not, she will route a note to the pod pool.
13. She postpones the shell for two weeks.
14. After two weeks, she looks through the chart and Care Everywhere to make sure that the patient has been scheduled.
15. If not, she calls the patient three times and then forwards to the provider for any additional follow up.

**Methotrexate**

Workflow Contact: Amy Palmer, Kayla Quinn

Teams Involved – Clinicians, Advice RNs, Clinical Team

Informed Consent for Methotrexate Treatment of Ectopic Pregnancy

A patient receives methotrexate:

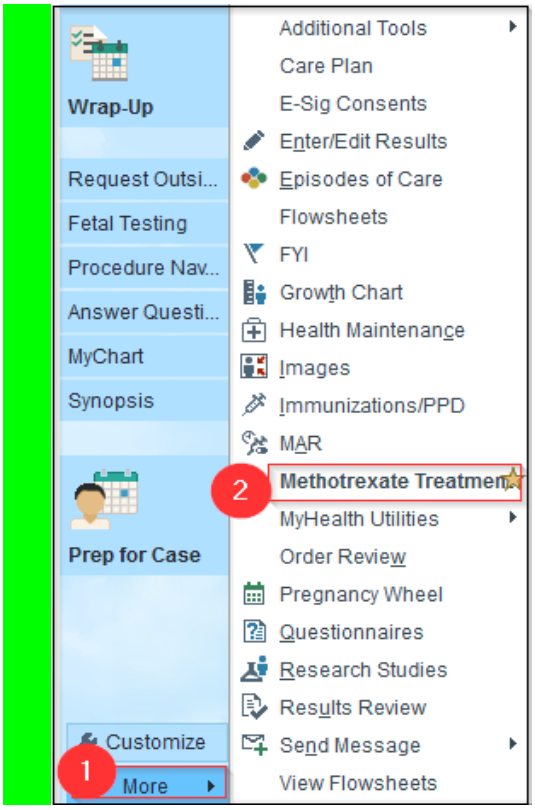
- **The clinician sends a staff message to the advice pool to notify them.**
- The advice RN starts the methotrexate treatment follow-up note in a telephone encounter. https://whallc.sharepoint.com/Epic/Tip%20Sheets/RN%20Methotrexate%20Workflow.pdf#search=methotrexate
- The RNs keep the follow up encounter open and monitor the bhcgs and coordinate all follow up care.
- The encounter is not ended until the loss is verified to be complete.

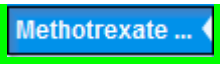
**MD Methotrexate Medication Activity**

Objective – To show how to access the appropriate activity, complete the Inclusion Criteria, and place the Methotrexate medication order.

**Access the Methotrexate Treatment Activity:**

1. Click the **More** button.
2. Select **Methotrexate Treatment** activity.
   a. It is suggested to click the star to make this activity a favorite for future reference.

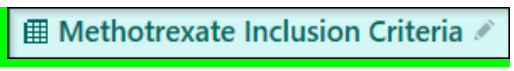



Click on the Methotrexate 

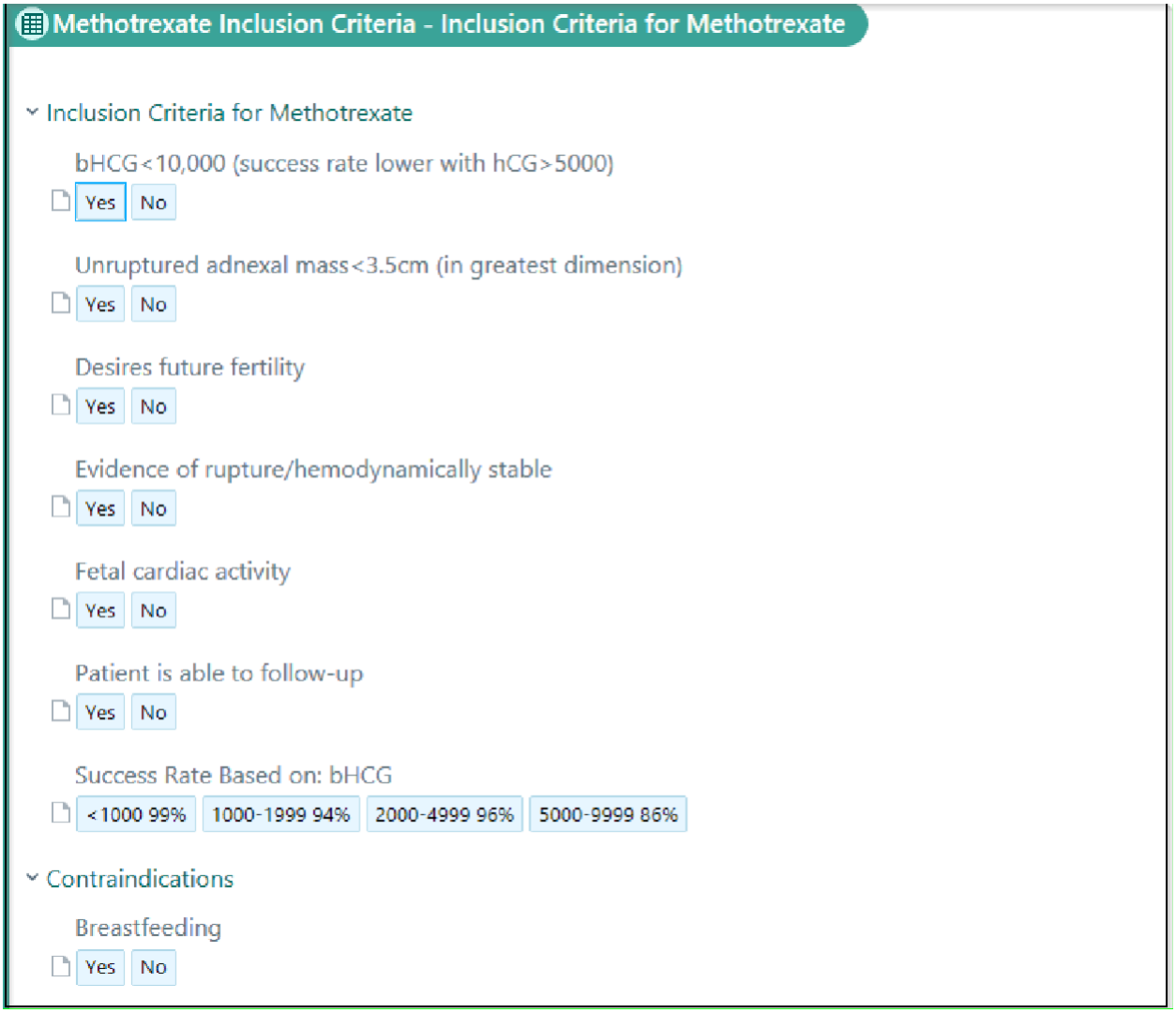
activity located in the left navigation pane.

**Methotrexate Inclusion Criteria – Counseling:**

Assuming the HCG, CBC, CMP, and Blood Type orders have been placed and resulted, and informed consent has been signed by the patient.

1. Click to open the **Inclusion Criteria** 

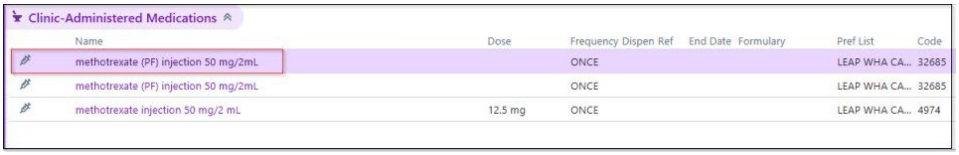
section.
2. Document the appropriate answers.

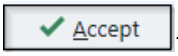


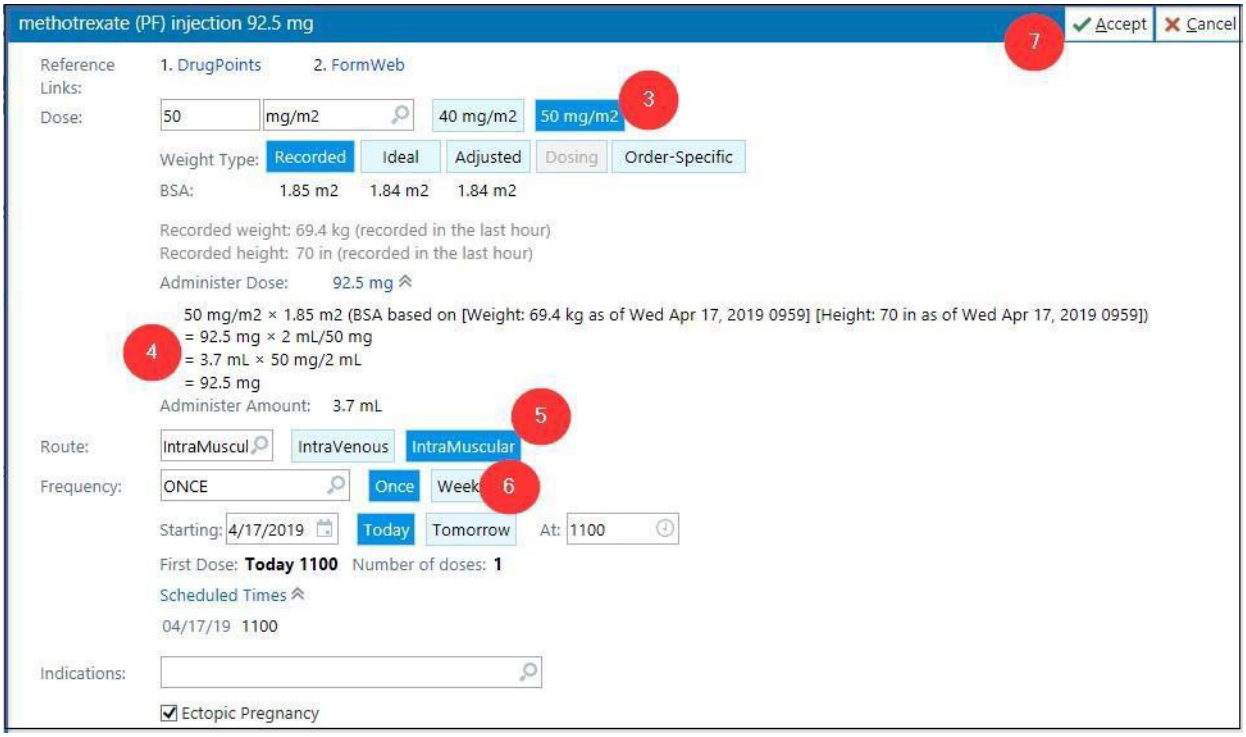

3. Create a staff message to the Advice pool for Follow Up.
4. Inform your call group, as well.

**Place the Methotrexate Injection Order**:

1. Click the **Orders Preference**  icon, search for the **Methotrexate** medication.

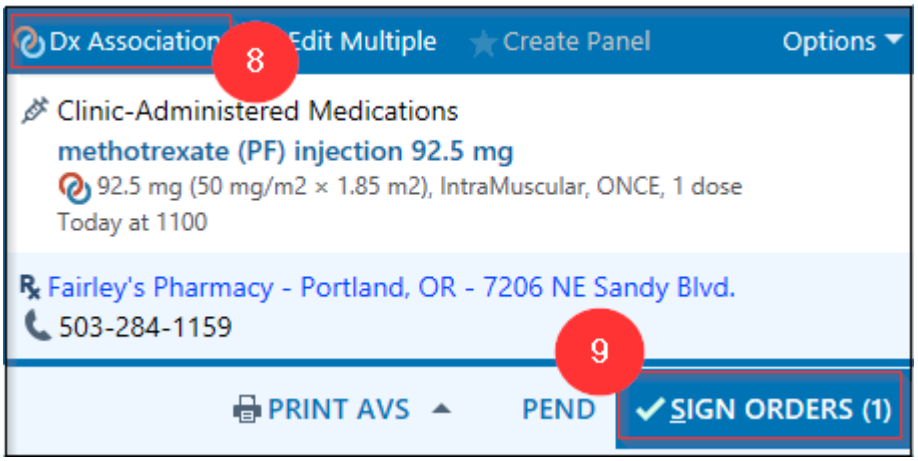



2. Click **Accept**  then openprescription.
3. Select the correct **Dose**.
4. Verify correct calculated **Administer Dose.**
5. Select the correct **Route.**
6. Update the **Frequency** according to the plan
7. Click **Accept**.

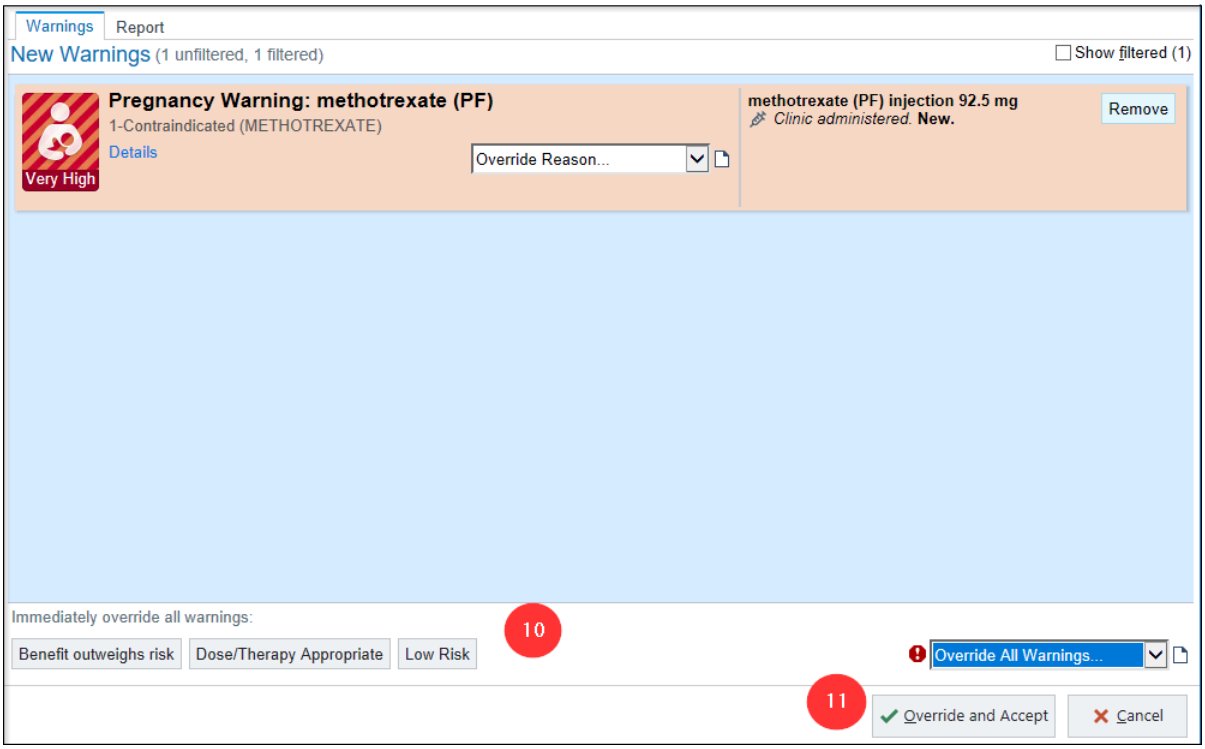



Option to update to a **Standing Frequency** to save orders for future release.

8. **Associate** the order with correct diagnosis.
9. Click **Sign Orders.**

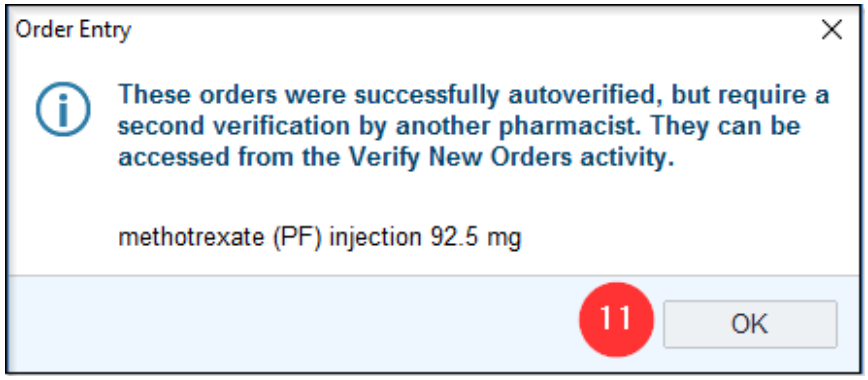



10. From the **Medication Warnings** window, select the appropriate **Override Reason.**
11. Click **Override and Accept**.



The **Order Entry** window, is a reminder that a **second verification** will be required with administration

12. Click **OK.**



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**Medication Prior Authorization**

Workflow Contact: Kristina Wood

Teams Involved – Clinicians, Clinical Team

All prior authorizations need to have a standardized way of completing to ensure they are being completed in a timely manner.

**Workflow:**

- MA/LPN receives fax or notification that Prior Authorization (PA) is needed.
- MA/LPN to complete the PA through Cover my meds. If no account previously set up, this should be set up through the cover my meds website (See supervisor for support/instructions).
    - MA/LPN may also call insurance to obtain PA if more time sensitive answer is needed
- Create a telephone note indicating that PA has been started and route to responsible provider clinical pod pool.
- Check for PA response Q2-3 days from submission.
- Once PA response has been received, update Telephone encounter:
    - **Approval** - call pharmacy to inform of the decision so they can fill patient's medication.
    - **Denial** - Route encounter to prescribing provider to advise if appeal should be attempted, if alternative med can be prescribed, or other next steps. MA/LPN to carry out provider's plan.
- Call to patient letting them know PA decision and next steps. Once phone note is complete and updated with PA information, patient has been notified please sign the phone note.
- Label PA authorization/denial if on paper and send for scanning.

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# Post-Op Calls

Workflow Contact: Kayla Quinn, Amy Palmer

Teams Involved – Clinicians, In-Office Surgery Coordinators, Surgery Scheduler, Advice RNs

## Hospital Cases

**Surgery Scheduler:** Once surgery is scheduled the surgery scheduler will send a staff message to the corresponding site's Advice Pool with the date of surgery.

**RN's:** RN to postpone staff message to day after scheduled surgery. If surgery is scheduled for a Friday. RN to forward & postpone staff message to Extended Hours Advice Pool to be completed on the Saturday following the scheduled surgery.

On the day after surgery. The RN will look in the chart or care everywhere to determine if the patient has been discharged.

a. If patient has been discharged, the RN will convert the staff message to a telephone encounter & call the patient and pull in the smart phrase, .WHAPHONEPOSTOPHOSPITAL if the patient is reached.
    I. **Patient reached:** If patient has post op concerns, the primary or assisting providers are the preferred clinicians to consult. If neither of them are available, another MD/Surgeon (or on-call MD if after hours) should be consulted if urgent and unable to wait until primary surgeon available.
    II. **Patient was not reached**: A voicemail should be left advising patient to return call if she has any post op questions or concerns. Only 1 initial phone attempt is needed.
b. If patient has not yet been discharged, the RN will postpone the staff message to the following day to recheck to see if patient has been discharged and call at that time.
c. Primary surgeon should always be CC'd the encounter to close the loop, especially if another clinician was consulted for an urgent need, or if the

patient was sent to the ER for a problem, but including if the patient was not reached or is doing well.

**IOS Cases**

**IOSC:** IOSC to keep track of scheduled/Completed IOS cases and will complete the next day post op calls using either .WHAPHONEPOSTOPIOS or .WHAPHONEPOSTOPIOSD&C depending on the type of IOS case.

a. If patient is reached and has Post Op concerns, she will be transferred to the advice queue to further assess patient and consult with surgeon PRN, and route encounter to Surgeon upon completion to close the loop.
b. If patient is reached and is doing well, has no concerns, IOSC will route encounter to surgeon upon completion to close the loop.
c. If patient is not reached, the IOSC will route encounter to surgeon to close the loop.

**Surgeon/Clinician (Hospital or IOS cases) –** Upon receiving the Post Op Call encounter, if you would RNs or IOSC to attempt to reach patient again (if patient wasn't reached) or ensure additional follow up occurs, document this in the encounter, and route back to site Advice Pool and they will follow up with patient.

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**STI Process**

Workflow Contact: Kristina Wood

Teams Involved – Clinicians, Clinical Team, Advice RNs

Due to a shortage of COVID tests we are no longer offering self-collect vaginal GC/CT for patients.

**Workflow:**

If a patient calls to get in for STI testing offer them an in-person visit with a provider if there is availability same day or next day.

- If there is no in-person availability please schedule the patient for a GYN Tele Visit to do STI counseling.
- The provider doing the counseling will determine if GC/CT testing is indicated and if yes: will enter instructions in for the patient to have an in-person exam to get this swab collected.
- Reception team will call the patient to get the patient scheduled for an in person GC/CT swab collection and/or lab appointment for any other tests the provider has ordered.
- If there is no 15 minute opening (or 10 minute prior to August) on any provider schedule for GC/CT collection, please overbook the patient to get them in.
- If a Patient is being recalled for treatment of cure GC/CT 3 months post prior positive:
    - If patient has questions regarding this test please schedule a GYN televisit with a provider.
    - If patient has no questions and wants GC/CT swab only schedule as an in-person visit on provider schedule if there is time, if no available time overbook as swab only visit.

**Clinical Services:**

- If patient is overbooked: Re-enforce that this visit is to collect the swab and that they have been previously counseled by a provider via televisit. **DO NOT** ask if they have and questions or concerns for this visit as the patient has already had a visit with a provider.
- Room patient quickly by obtaining a set of vitals and asking them to change for the GT/CT swab collection.

If patient is on the schedule for counseling please room as normal.

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**Uncertain Pregnancy Viability Follow-up**

Workflow Contact: Kayla Quinn, Amy Palmer

Teams Involved – Clinicians, Advice RN, Clinical Team, OB Coordinator

- When clinician or nurse becomes aware of a patient with uncertain pregnancy viability, they should send a staff message to their local advice pool with a follow-up plan for the patient. The plan should be outlined in the visit or telephone note.
- The advice RN, monitoring the pools for that day, receives the message and initiates a phone note to document/follow the patient through pregnancy viability confirmation or loss.
- If the patient has a follow up ultrasound scheduled, the RN will ensure her ultrasound is completed at the correct time and that the provider updates the plan of care in the telephone note or in a scheduled visit.
- If the patient has Bhcgs scheduled, the RN will look for the results on the day they are to be completed and be sure that the responsible or covering provider gets the results and updates the plan of care.
- This process repeats until pregnancy IUP viability is confirmed, or the patient has a confirmed loss or ectopic.
- If the pregnancy viability is confirmed, the nurse will ensure a NOB visit is scheduled.
- If the Pregnancy Loss is confirmed:
  - Ectopic - RN will follow methotrexate protocol or ensure surgical management is scheduled.
  - MAB/SAB - RN will ensure patient has a follow-up visit with her provider to determine management options.

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**Well Being Assessment**

Workflow Contact: Adrienne Thompson, Kayla Quinn and Caitlin Mackenzie

Teams Involved – Clinicians, Behavioral health, Patient Services, Clinical team, Advice RNs

The WBA includes:

1. **A mood screener**. Either the EPDS (OB) or the PHQ-9 (Gyn). Scores of **13** or greater on EPDS and **15** or greater on the PHQ-9 send an alert to the location's behavioral health pool. Any yes to a suicidal ideation question sends an urgent alert to an RN and behaviorist pool. Any lower score will not be reviewed unless provider reviews with patient.

2. **Alcohol and drug use screeners.** SBIRT-qualifying screeners. AUDIT/DAST/CAGE (how much you use and how you feel about it) for Gyn and 5P's for OB (asks about personal use past and present and peer and partner use). **More than two affirmative answers** for risk factors sends an alert to the location's behaviorist pool.

   Also asks if OB patients have had trouble stopping alcohol and drug use since finding out about the pregnancy and if they are taking prescribed pain medications— **yes to either of these sends an alert to behaviorist pool**.

3. **Relationship safety.** Ob and Gyn. Do you feel safe at home? **A no answer sends an urgent alert to an RN and behaviorist pool**. Does your current partner talk to you in ways that make you feel bad? Does your current partner control where you go, who you talk to or how you spend money? Has your current partner hurt or threatened you or forced you to have sex? **A yes to 2 or more sends an alert to the behaviorist pool.**

4. **Parenting support.** Do you have the social and emotional support you need for parenting? **A no answer sends alert to behaviorist pool.**

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Appointment Types for which the WBA is administered:

1. OB Intake
2. New Patient
3. Wellness
4. Any of the above that occur over telehealth

If WBA is filled out online:

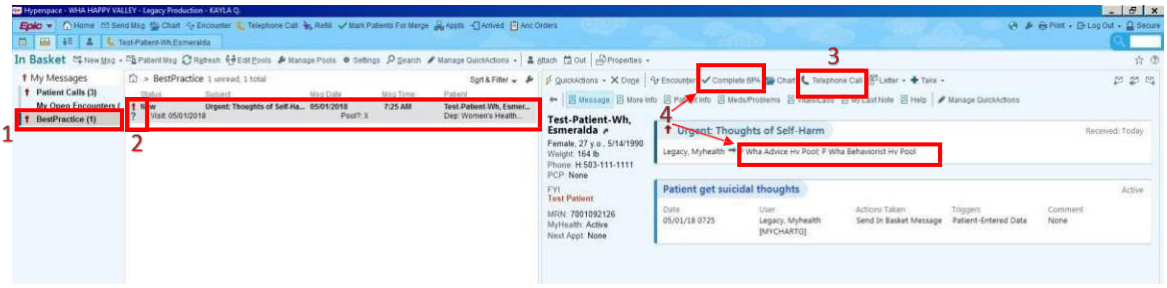
1. If the patient has an elevated or positive score on a safety issue (self harm, not safe at home) triggers epic to alert the advice and behaviorist teams.
    a. If the appointment is same day the RN will reach out to the MA/LPN or clinician to alert them.
    b. If the appointment is the next day or further out, the advice nurse will call the patient to assess, setting her up with the behaviorist or resources.
2. The MA will also look through the synopsis to review items that need to be addressed with the patient and update the clinician.
3. Clinician addresses these concerns making sure to document the discussion so that it is clear the discussion happened. This will let advice or the behaviorist know that they do or do not need to call the patient.

If the WBA is filled out in person:

1. Patient receives WBA when checking in to fill out while waiting to be called back.
2. MA/LPN pulls patient back and goes over WBA with patient, putting the PHQ-9 or EPDS into the chart via the screening activity
3. MA/LPN reviews questions about feeling safe at home, need for pregnancy and parenting support and alcohol and drug use
4. When giving report to clinician, makes sure clinician is aware of any positive triggers (Elevated PHQ-9/EPDS, alcohol or drug use or feeling unsafe or need for pregnancy and parenting support).

5. Clinician addresses these concerns making sure to document the discussion so that it is clear the discussion happened. This will let advice or the behaviorist know that they do or do not need to call the patient.
6. MA/LPN gives completed WBA to PS team, marking that the PHQ-9/EPDS was entered.
7. Patient Services team enters the WBA through "Answer Pt-Qnr" including re-entry of PHQ-9 or EPDS. *PS team does not re-enter Medical Histories.*
    a. If unable to enter a WBA same day, the WBA must be scanned for positive triggers (*see GYN or OB Triggers guide*). If positive trigger is present and there is no documentation that this was discussed in visit, please **call** your site's designated pool nurse.

Addressing WBA Triggers:




1. In the In-basket it will populate under its own folder of **BestPractice** as high priority as indicated by the red up arrow.

2. If multiple people are working out of a pool they should take responsibility by clicking on the **question mark** so others (who are signed in to your same pool) know **You** are working on that message and don't work on it (although the behaviorist wouldn't be able to know if an RN had taken responsibility for it, so there is the potential that two people could start a telephone encounter at the same time, always check if one already started).

3. On the right side of the screen you can click telephone call and bring in the **Columbia Suicide Risk Assessment Protocol** (always click View calls to see if someone already has an open telephone encounter for same topic).

4. You can see that the **BestPractice Alert** was sent to both the HV Advice pool, and the WHA Behaviorist HV Pool. Once this has been addressed, you can click **Complete BPA**, and it should remove the alert from both the Advice pool & the Behaviorist pool.

**** Do Not complete the BPA until you have started the telephone encounter, otherwise it will disappear from both the pool and could get lost****

Telehealth and the WBA:

The WBA is also administered for telehealth visits (new patient, wellness, OB intake can all occur via telehealth).

If the patient doesn't fill it out prior to telehealth appointment the workflow is for the MA to verbally complete it with them and enter responses during the rooming process. In this scenario the provider may not be alerted to any issues prior to the visit though ideally the MA would alert them. **Providers can view the WBA through the synopsis view in the patient's chart.**

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# FMLA Process

Workflow Contact: Carissa Morgan

Teams Involved – Clinicians, Clinical Teams, Patient Services

## Site Receives FMLA / Leave Paperwork:

1. Collect ROI from patient. Collect FMLA processing fee. Start phone FMLA phone note and route to the hub pod pool
2. Fax or walk paperwork to FMLA inbox at the hub

## Hub Location Receives Paperwork:

1. MA updates phone note for tracking
2. MA completes FMLA/leave paperwork to WHA standard
3. MA stores completed but unsigned FMLA in designated area of pod
4. Prompts the provider physically sign completed paperwork (cosign for their partners if applicable)

## Process for Completing and Signing Paperwork:

1. MA calls patient to inform of completion and confirm email address, employer fax number, or physical address in epic.

**a. Email Return to Patient PREFERRED:**

i. Email completed forms with WHA cover sheet to patients confirmed email address directly through the fax machine 8/5/2020 2

ii. Print patient labels in Epic and affix to upper right hand corner of all original pages

iii. Send to HIM for scanning

iv. Document steps 1-4 in FMLA phone note found in patient's chart and sign note

**b. Fax Directly to Employer:**

i. Confirm ROI is on file for employer

ii. Fax completed forms

iii. Print patients labels in Epic and affix to upper right hand corner of all original pages

iv. Send to HIM for scanning

v. Document steps 1-4 in FMLA phone note found in patient's chart and sign note

**c. Mail Return to Patient:**

i. Make a copy of completed FMLA / leave documents

ii. Place copied document into WHA envelope and seal. Affix patient address label to front and place in outgoing mail pile

iii. Print patient labels in Epic and affix to upper right hand corner of all original pages

iv. Send to HIM for scanning

v. Document steps 1-4 in FMLA phone note found in patient's chart and sign note

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# Insulin Teaching

Workflow Contact: Suzanne Adlesich

Teams Involved – Clinicians, NWP CCC, NWP Nurses, NWP Patient Services, Advice Nurses

**Workflow:**

- WHA Clinician who prescribes insulin places NWP MFM Referral - 30 min Nurse Visit appointment at NWPW. This automatically routes to the CCC.
  - Generalist OB (if APC must consult with OB at HUB) determines starting insulin dose.
  - Insulin, insulin syringes and sharp's container prescriptions are sent by the referring clinician to the patient's pharmacy. Insulin syringes and sharp's container may be considered DME and may need to be sent to DME Pharmacy by Advice RNs once they have spoken to the patient's insurance company.
- WHA Clinician not comfortable with prescribing insulin may request MFM Consultation by placing referral for a MFM Consultation which routes to the NWP CCC.
  - Consult may be scheduled either In Person or by TeleHealth.
  - NWP MD to order insulin and supplies.
  - NWP RN to teach insulin administration either at time of appointment or scheduled after TeleHealth Visit.
- The long term plan is to work towards training RN's at each Hub for the insulin teach.

**Makena**

Workflow Contact: Suzanne Adlesich, Kayla Quinn, Amy Palmer

Teams Involved – Clinicians, NWP Nurses, NWP Financial Counselors, Patient Services, Clinical Team, and Advice Nurses

Clinician requests patient starts on Makena by sending staff message to MA/LPN pod pool. If the occurs at NWP, this note is sent to the advice pool.

A telephone note is kept open in the pod/advice pool where multiple correspondence will be added and tracked including PA's, record requests, and shipping information. The phone note is left open until the medication arrives at the clinic and the patient is notified. Any follow-up that needs to occur can be through the phone note being postponed.

1. The MA/LPN calls the patient's insurance to find out the preferred specialty pharmacy, whether a PA is needed and if PA cannot be done over the phone while you have insurance on the line, what form needs to be completed for the PA. (At NWP the financial counselor does this piece.)
2. Clinician (or RN at NWP) enters in the prescription to the chart (as a pharmacy prescription and NOT clinically administered)and MA/LPN/RN sends to the preferred specialty pharmacy per patient's insurance and keeps the phone note open until the first rx arrives on site for tracking progress, and to address all the potential communication prior to arrival.
    a. Typically the Rx is started between 16 weeks-20 weeks gestation and continued through 36 weeks which is a total of 16-20 weeks/administrations. The rx for the pharmacy will be the generic Hydroxyprogesterone Carpoate 250mg/mL IM OIL weekly injection Rx with a quantity of **2-4/1 ml doses** and generally with 8 or 4 refills. (This prescription is found by looking in the database.) OHP sends 2 doses at a time and commercial insurance will send 4 doses at a time. It's a good idea to determine what the patient's DME pharmacy will dispense prior to sending in the prescriptions and the clinical team will do that work.

   b. Once the medication has arrived determine if the patient will be receiving clinic administrations or will have a family member taught for home injections.
      i. If home injections, obtain Support Person Approval from Command Center for Nurse Visit for teaching.
      ii. When giving the injection add Makena, not the generic, to the MAR by choosing a Clinical Administered Medication (needle and syringe sign by Makena and on the preference list) and the number of doses to be administered at WHA.
3. RN/LPN/MA communicates with on-site lab assistant/MA/LPN to make sure they are keeping track of patient's Makena doses and reorder from DME pharmacy as appropriate. Reorders should happen when the patient has one vial left. Lab assistants/MAs should check chart to ensure the patient hasn't delivered prior to reordering.
4. If a patient transfers back to generalist care from NWP while receiving Makena treatment, the NWP nurse will send a staff message to the patient's sites pod pool #1 to be further managed at the new site.
5. When a patient delivers, we need to cancel any remaining prescriptions.

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# Mifepristone for Medical Abortion

Workflow Contact: Amy Palmer, Ashley Bautista and Kristina Wood

Teams Involved – Clinicians, Clinical Services

**Candidates for Regimen:** Patient with an undesired pregnancy that is 9w6d or less. If patient has a reliable LMP and menses are regular, no dating US is needed. Clinician should consider US order if dating is uncertain.

**Exclusion Criteria:**

- Unable to document intrauterine pregnancy or concern for ectopic pregnancy
- Known severe anemia with HGB <9.5
- An IUD in place (Must be removed if patient desires to proceed.)
- Current long-term systemic corticosteroid therapy
- Chronic adrenal failure
- Known coagulopathy or anticoagulant therapy
- Intolerance or allergy to mifepristone or misoprostol
- Severe liver, renal or respiratory disease, uncontrolled hypertension, cardiovascular disease
- Patient unwilling or unable to adhere to care instructions, not available for follow-up contact*, cannot understand instructions due to language or comprehension barriers.

Workflow Details:

- Patient chooses medical management of her undesired pregnancy in an options counseling visit. Mifepristone can be administered at that time at **TN, GW, BR, HB, PK**. If patient is being counseled at OC, NB, or TA patient will need to be scheduled for 30min visit with a mifepristone provider at alternative location for administration.
- Medication abortion does **not** require dating US if reliable LMP and menses regular.
    a. Clinician should consider US order if dating uncertain; clinicians may use judgement and shared decision making with patients regarding these orders.
    b. ABO Rh and ABC are not required for medication abortion; clinicians may use

judgement and shared decision making with patients regarding these orders.

- Once patient is deemed appropriate for Mifepristone, the medication may be administered by the clinician. Clinical staff can obtain medication from their site designated team member (supervisory staff) to hand-off to the clinician with the consent form and patient information packet.
- **Clinician will utilize smart set for diagnosis, orders, charting template, AVS Smart phrase, and nursing follow up referral for therapeutic abortion:**

**This is an example of the smart set Medication Regimen orders:**
Mifepristone 200mg PO x 1 dispensed by WHA and taking it is witnessed by the provider in the clinic.
Misoprostol 800mcg prescription sent to the pharmacy for patient to take 24hrs after mifepristone bucally or vaginally. (can be taken up to 72 hours later)
Rhogam is ordered and given, if necessary, when mifepristone is given.
Anti-emetics and pain medication are chosen from the smart set and sent to the pharmacy.

- PATIENT MUST SIGN DANCO "PATIENT AGREEMENT FORM" FOR MIFEPREX.
- The patient is sent home with an information packet and a copy of the agreement form given by the support staff.
- Here is the link to the "Patient Agreement forms" and education materials to review.

  https://www.earlyoptionpill.com/for-patients/download-mifeprex-materials/

**Follow-up**:
Acceptable ways to confirm pregnancy has expelled:

a. Patient's report of bleeding. When patient and clinician both feel expulsion has happened based on symptomatology, they are correct 96-99% of the time.
b. Transvaginal ultrasound demonstrating GS is absent
c. HCG drop of >/=80% in one week

d. Negative UPT 4 weeks after AB

e. Contraception (if applicable): IUD can be placed one week after SAB/MAB. All other methods can be started as soon as the day of misoprostol administration.

- Recommended Follow up during COVID Crisis:
  a. Provider will use smart set to initiate recommended follow up protocol.

Workflow Details for Clinical Services:

1. Room patient in standard fashion.

   a. Chief Complaint – "Other" In comments add, therapeutic abortion.

   b. Progress note – Pull in GYN problem visit.

      i. Provider must utilize Smart Set to pull in Diagnosis, orders, charting template, nursing referral, and AVS.

         1. Patient will take Mifeprex in office during visit.

2. Consent Form should have patient label placed in upper right hand corner. Once patient has signed the consent form, a copy should be given to the patient and the original should be sent to scanning.

3. Patient should be sent home with Mifeprex medication guide, patient brochure. https://www.earlyoptionpill.com/for-patients/download-mifeprex-materials/

4. MRN added to log sheet to track and reconcile Mifeprex counts.

Workflow Details for Advice:

Utilizing the smart set correctly will initiate a nursing referral order for follow up as desired. These are managed by the RNs out of their advice referral work queue.

# Medical Abortion (GA up to 9w6d)

Patient calls in with i:;ossibility of wanting to terminate pregnancy

Options counseling visit with a provider- can be telehealth

Patient decides to terminate pregnancy and selects medical abortion

Appointment scheduled to receive medication and sign consent

Patient romes in to the clinic for app::iintment

Initiate RN referral order for follow up (in person, telehealth or phone call)

**Medication abortion does not require dating US if reliable LMP and menses are regular. Clinician should consider US order if datinguncertain. ABORh and ABC are not required for medication abortion; clinicians may use judgement and shared decision making with patients regarding theseorders.**

In MIF packet:

- Mifepristone 200mg PO x 1 dispensed.
- RX for Misoprostol 800mcg buccal to take 24hrs after mifepristone lean be taken up to 72 hrs later)
- Mifeprex Information sheet
- PATIENT MUST SIGN DANCO "PATIENT AGREEMEI'JT EORM" FOR MIEEPREX E'iEN IE SAB

Home pregnancy test four weeks post Mif

Patient calls in with i:;ossibility of wanting to terminate pregnancy

Options counseling visit with a provider- can be telehealth

Patient decides to terminate pregnancy and selects medical abortion

Appointment scheduled at other site to receive medication and sign consent

**Medication abortion does not require dating US if reliable LMP and menses are regular.**
Clinician should consider US order if dating uncertain. ABORh and ABC are not required for medication abortion; clinicians may use judgement and shared decision making with patients regarding these orders.

Exclusion Criteria:

- Unable to document intrauterine pregnancy or concern for ectopic pregnancy
- Known severe anemia with HGB <9.5
- IUD in place (can be removed and proceed)
- Current long-term systemic corticosteroid therapy
- Chronic adrenal fa iiure
- Known coagulopathy or anticoagulant therapy
- Intolerance or allergy to mifepristone or misoprostol
- Severe liver, renal or respiratory disease, uncontrolled hypertension, cardiovascular disease
- Patient unwilling or unable to adhere to care instructions, not available for follow-up contact*, cannot understand instructions due to language or comprehension barriers.

Other Meds Consideration

Give ibuprofen 600mg PO q6hrs prn !*advise patient not to take within 1 hour of using misoprostol), can give oxycodone or other narcotic as well at provider discretion.
Rhogam mini-dose as appropriate
Zofran prn at time of mil administration (if having nausea, might throw up expensive tablet) and for home.

# IV Infusion

Workflow Contact: Suzanne Adlesich

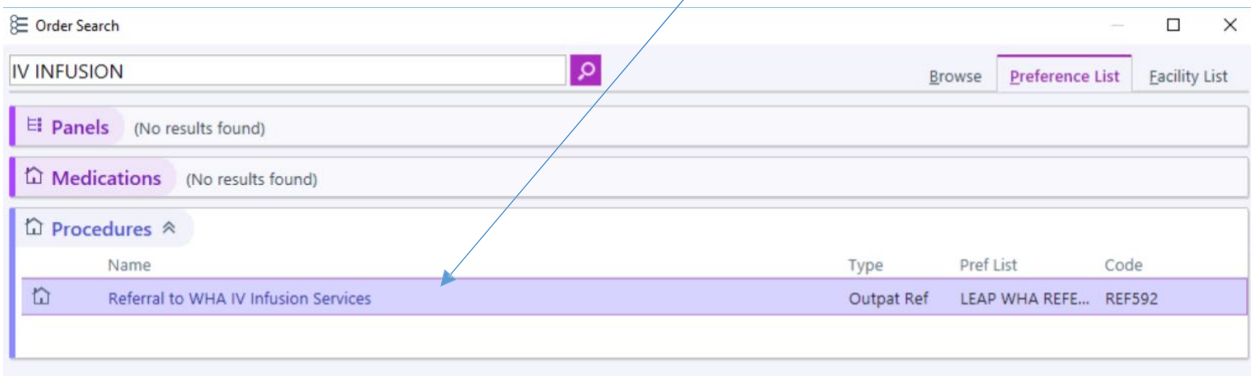
Teams Involved – Clinicians, Advice RNs, Patient services

Patient calls to report nausea and vomiting in early pregnancy and a telehealth appointment is scheduled.

Clinician completes telehealth visit and confirms:

- Patient has documented IUP – (If no documented IUP ultrasound is ordered prior to referral)
- Patient's nausea and vomiting had a 1st trimester onset
- Patient failed oral therapy and conservative measures
- Patient exhibits signs of dehydration.

Clinician places electronic order as shown below




NWP Clinical Care Coordinator receives order and schedules patient.

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# Diabetic Education Referral

Workflow Contact: Amy Palmer and Kayla Quinn

Teams Involved – Clinicians, Clinical Team

**Workflow:**

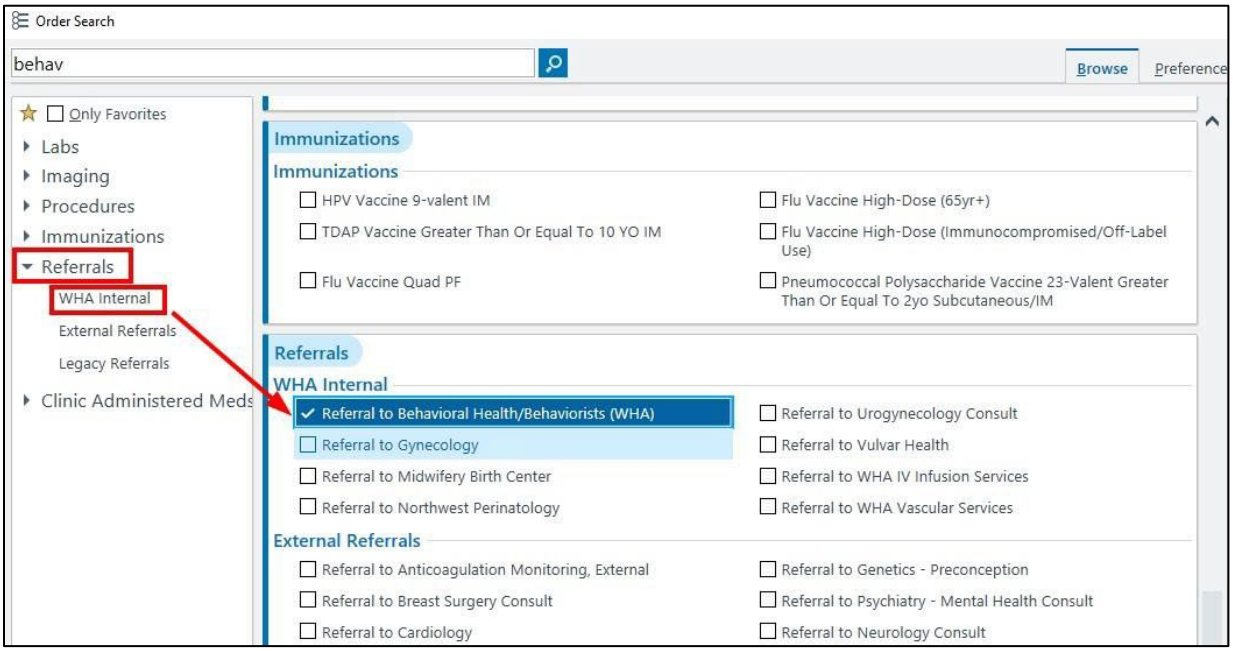
- Clinician identifies need for diabetic education based on diagnosis of gestational diabetes.
- Clinicians send a staff message to the pod pool.
- MA/LPN (RN at NWP) completes paper referral for providence and electronic for Legacy.
- MA/LPN calls patient back in one week to make sure they have had, or do have, an appointment with the diabetic educators.

**Referring to Behavioral Health**

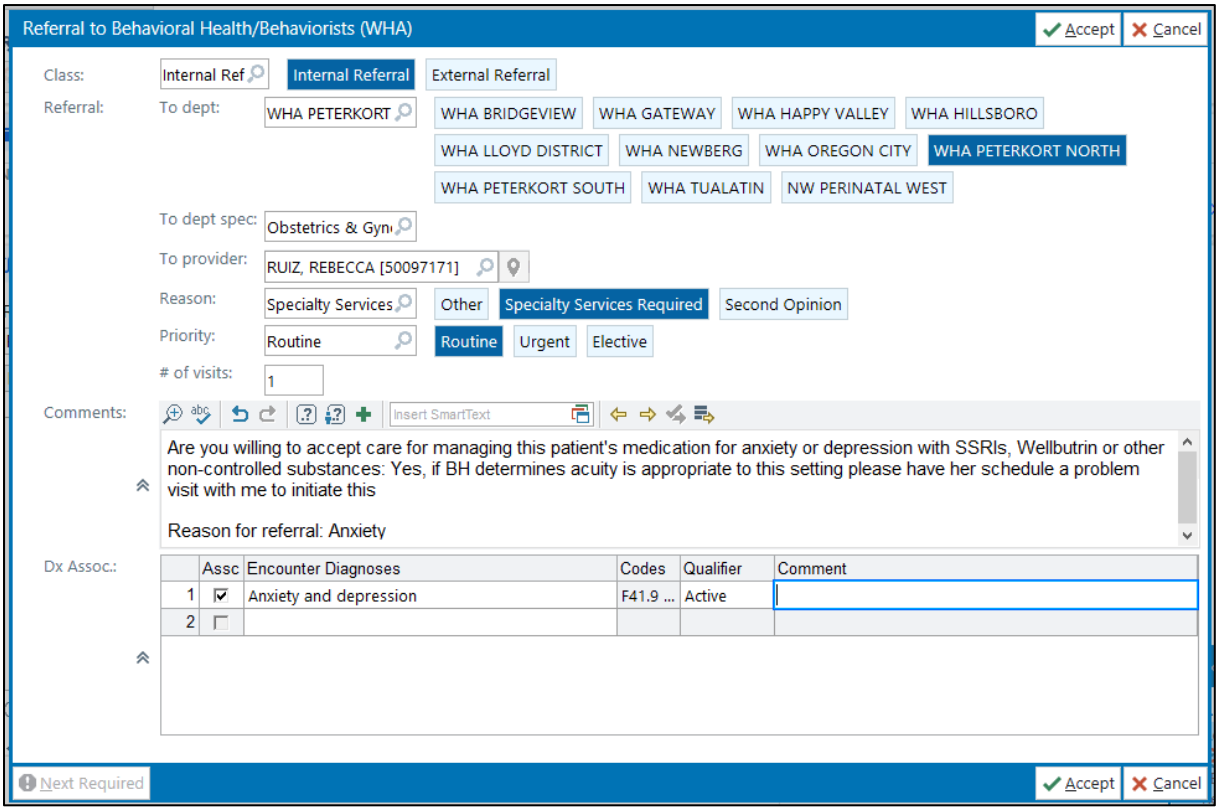
Workflow Contact: Jill Bozek

Teams Involved – Clinicians

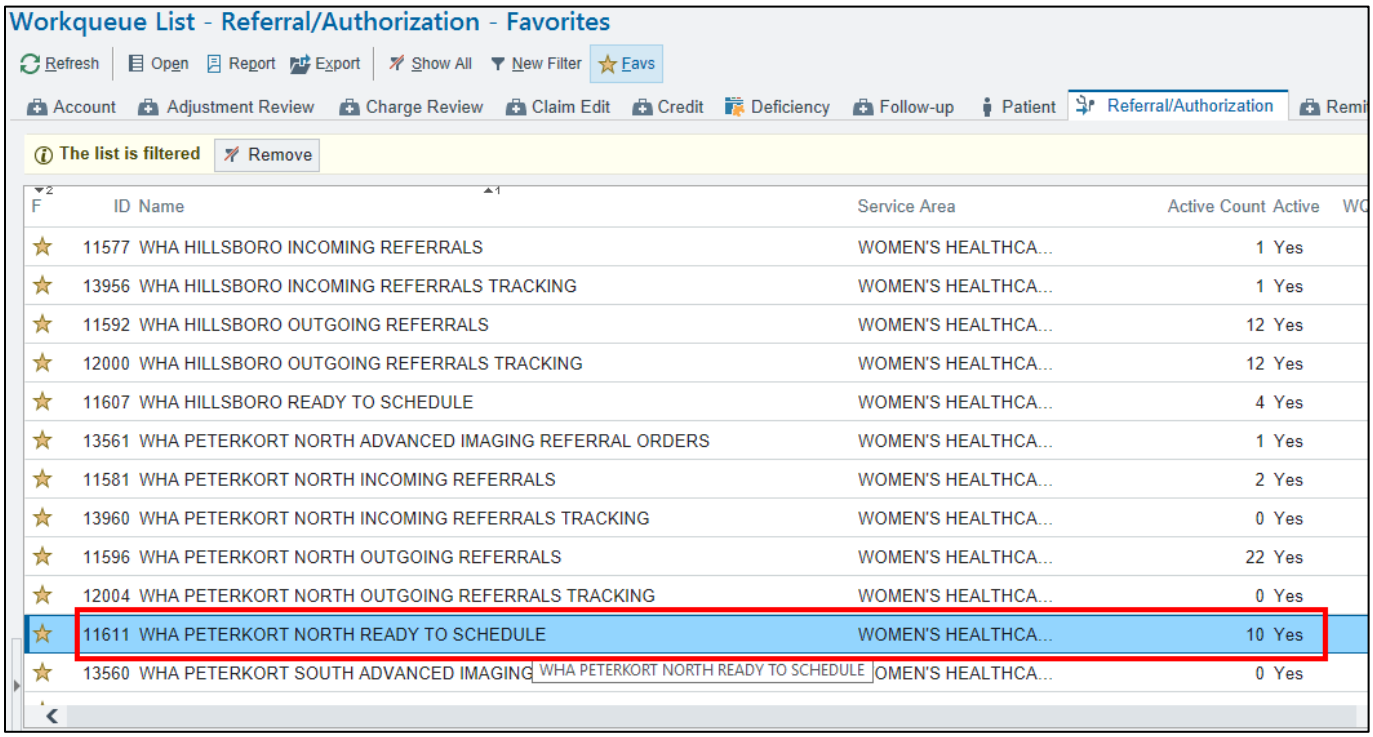
1. **Clinician** enters Referral to Behaviorist via Orders in patients chart



2.

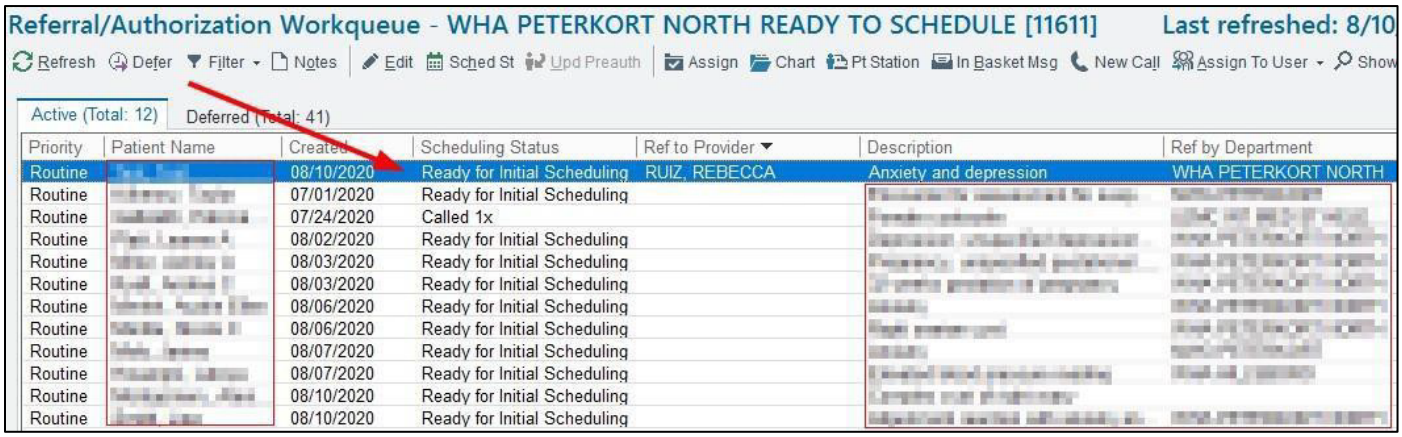

Referral to Behavioral Health/Behaviorists (WHA)
Accept
Cancel
Class:
Internal Ref
Internal Referral
External Referral
Referral:
To dept:
WHA PETERKORT
WHA BRIDGEVIEW
WHA GATEWAY
WHA HAPPY VALLEY
WHA HILLSBORO
WHA LLOYD DISTRICT
WHA NEWBERG
WHA OREGON CITY
WHA PETERKORT NORTH
WHA PETERKORT SOUTH
WHA TUALATIN
NW PERINATAL WEST
To dept spec:
Obstetrics & Gyn
To provider:
RUIZ, REBECCA [50097171]
Reason:
Specialty Services
Other
Specialty Services Required
Second Opinion
Priority:
Routine
Routine
Urgent
Elective
# of visits:
1
Comments:
Insert SmartText
Are you willing to accept care for managing this patient's medication for anxiety or depression with SSRIs, Wellbutrin or other non-controlled substances: Yes, if BH determines acuity is appropriate to this setting please have her schedule a problem visit with me to initiate this
Reason for referral: Anxiety
Dx Assoc.:
Assc
Encounter Diagnoses
Codes
Qualifier
Comment
1
Anxiety and depression
F41.9 ...
Active
2
Next Required
Accept
Cancel

3. Referral falls into *WHA PETERKORT NORTH READY TO SCHEDULE* referral work queue

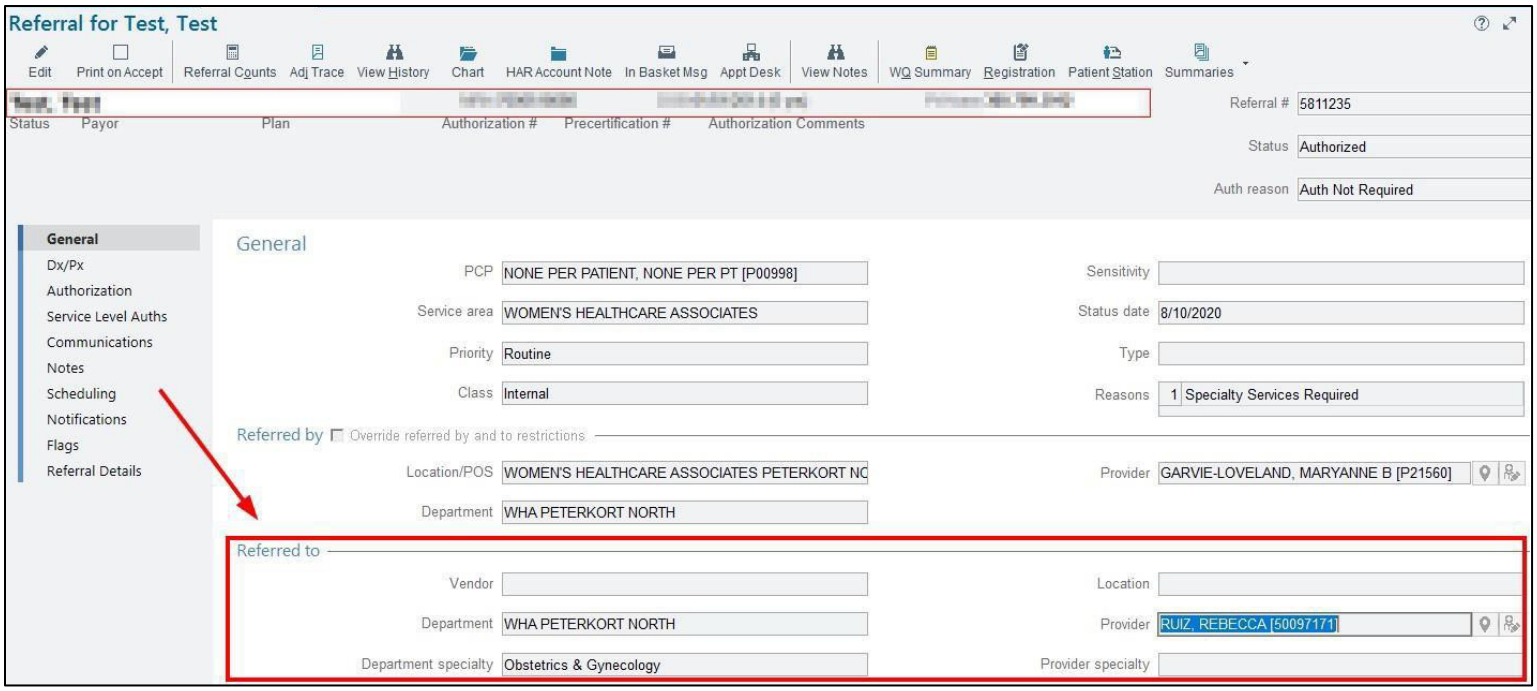


**Referral Navigator** enters work queue and selects referral

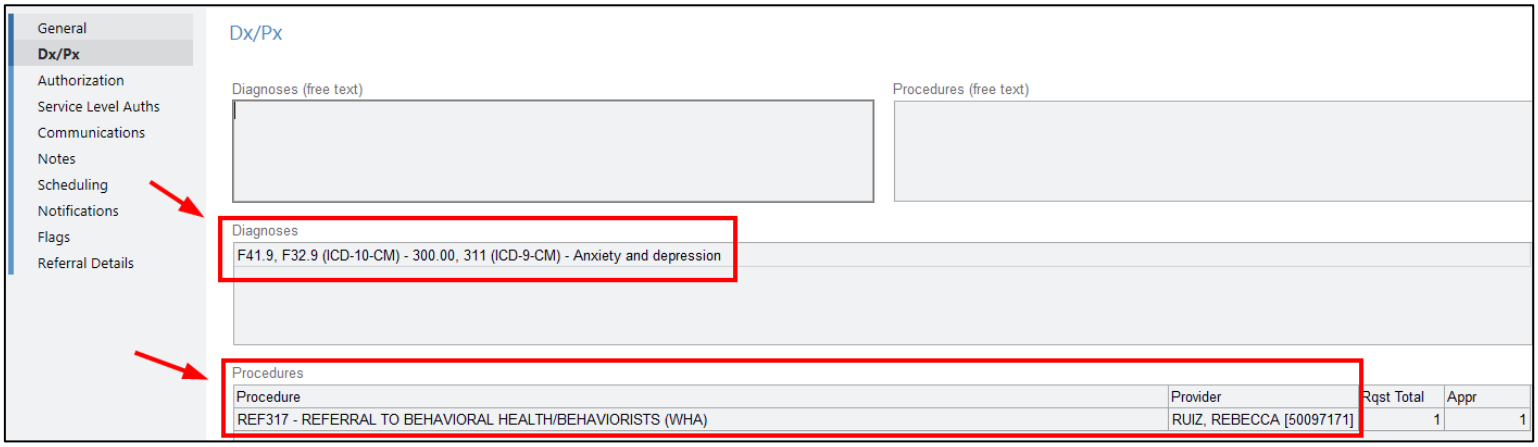
1.




2. Referral Navigator sees who patient is being referred to

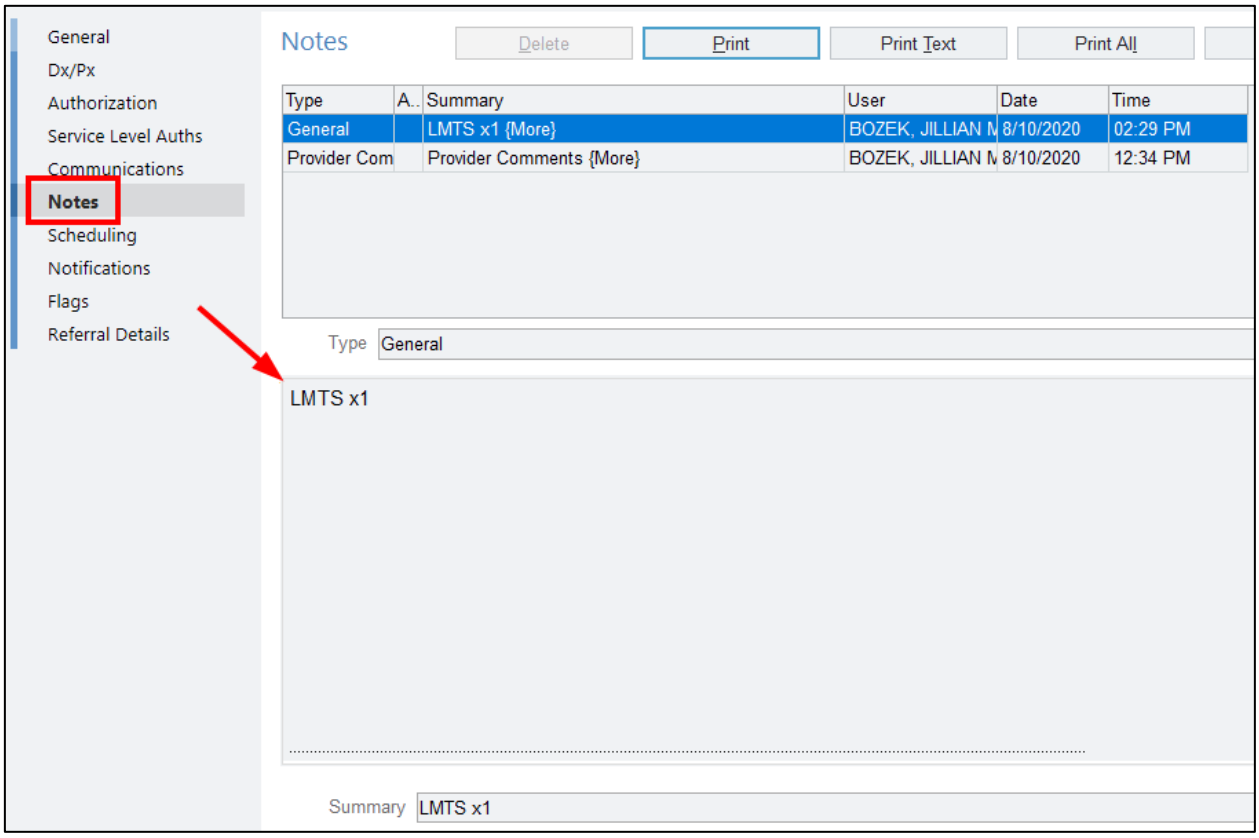


3. AND what the patient is being referred for [Diagnosis]




4. Referral Navigator calls patient to schedule and documents each attempt






5. If Referral Navigator is unsuccessful at scheduling patient, they send a staff message to BH Pool informing them patient has been contacted (or if patient declines to schedule).

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# Postpartum Call

Workflow Contact: Caitlin MacKenzie and Kristina Wood

1. Designated PS staff member runs delivery report and sends staff message to the Pod Pool alerting them of delivery.
2. MA/LPN creates telephone encounter at one week postpartum. MA pulls in the quick text .WHACOVID1WKPPCALL to assist in screening patient for need for in person visit.
3. MA/LPN reviews chart to look for delivery report and complications. They then update OB history for patient.
4. MA/LPN calls patient and asks the questions outlined in the template. (See template below.)

**1wk Postpartum Follow-up Phone Call**

Prior to calling Adella {chart review / Care Everywhere updated:38117}.
Adella had a {Delivery Type:33204} delivery and per chart review newborn was {d/c home / NICU:38121}. Her pregnancy history was updated with delivery & newborn data.
Per chart review, Adella had the following complications during her delivery or postpartum: {complications delivery:37878}
Adella was called to follow up and see how she is doing postpartum.

{PP Call LMSG /Reached patient:38620}

***How are you adjusting to being home? ****
***How are you feeling emotionally? ****
***Are you having any pain? ****
***Any problems with your incision/laceration (if patient had 3rd degree laceration, and currently doing well, made aware we will re-assess day of her appointment and if having any concerns will schedule an in-person visit)? ****
***Are you urinating Okay? ****
***Have you had a bowel movement? ****
***Do you have any questions or concerns about your bleeding? ****
***Are you breastfeeding? {Breast feeding yes no:38122}***

***Have you already been in touch with your pediatrician to schedule newborn visit?: ****
***3wk Postpartum Visit Scheduling:*** **{3wk PP Visit scheduling:37877}**

5. Upon completion of call, MA/LPN initiates a warm transfer to get patient scheduled for postpartum 3 week visit. If unable to reach scheduler, MA/LPN will send a staff message to the Patient Services pool to call and get patient scheduled.
6. If patient doesn't answer when MA/LPN calls, MA/LPN will leave a voicemail for patient. **"Hello, this is *** calling from Women's Healthcare Associates. I am calling to check on you after your delivery. If you haven't scheduled your 3 week postpartum visit, please call *** to get that scheduled. If you have any questions or concerns, our advice team is ready to help! Have a great day, and congratulations!"**
    **a)** F2 to complete **{PP Call LMSG /Reached patient:38620}** portion of the smart phrase, and delete the remainder of the smart phrase if patient doesn't answer. This will document the type of PP visit patient needs based on your chart review (in person vs. telehealth) to help guide patient services if the patient calls back just to schedule her PP visit.
        I. If you do not delete the remainder of the smart phrase you will not be able to sign your note.
7. MA/LPN calls patient one time and if patient doesn't return call, MA/LPN to close telephone encounter in the pool after 24 hours.

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# Manual Vacuum Aspiration In-Office Protocol

Workflow Contact: Amy Palmer, Kayla Quinn, Ashley Bautista

Teams Involved – Clinicians, Clinical Teams

Uterine aspiration for miscarriage management, retained products of conception, or induced abortion up to 9 weeks 6/7 days gestation

**Premedication to be prescribed by clinician, and to be taken by patient 30-45 minutes prior to procedure:**

- Ibuprofen 600mg PO
- Ativan 1mg PO
- Norco 2 tabs PO

**Procedure Setup:**

- Speculum
- Single tooth Tenaculum
- 10mL 1% lidocaine in control top syringe with 22g spinal needle
- Cotton swabs (3) with betadine or Hibiclens
- Ring forceps
- Dilators
- MVA
- Cannulae
- Sterile bowl
- Ultrasound
- Glass pan
- Backlight

**Patient Care:**

- Confirmation of gestational age by ultrasound will be performed unless recently confirmed by a provider.

- In-office procedures will be performed under local anesthesia with 1% lidocaine and premedication with PO Ativan, Norco and NSAID.
- Patient's blood type must be confirmed; Rh immune globulin will be administered to all consented Rh negative patients unless they received it already during this pregnancy.
    - If Rh status is unknown, it can be drawn the same day; patients should receive Rh immune globulin as soon as possible (within 3 days) if they are found to be Rh negative.
- Miscarriage patients will be offered genetic testing.
- All patients will receive family planning counseling. If a patient desires an intrauterine device for long-term contraception, it can safely be placed immediately after the procedure.
- Methergine and misoprostol will be available for cases of uterine atony.
- All products of conception will be sent to pathology.
- Counseling will include instructions on aftercare, symptoms, precautionary measures, behavioral health handout and a 24/7 emergency number to contact.

**Aftercare:**

- Doxycycline 200mg PO (Metronidazole 500mg BID x 3 days if allergy) to take at next meal after procedure.
- Can take ibuprofen 600mg PO q 6 hours and/or Tylenol 1000mg PO q 6 hrs prn
- Narcotic prescription per provider discretion
- Contraception

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# Lupron Ordering

Workflow Contact: Caitlin MacKenzie and Kristina Wood

Teams Involved: Clinicians, Clinical Teams

1. Clinician determines a patient is a candidate for Lupron and sends a message with specific instructions on dosing (1 month or 3 month dose) to the pod pool to initiate workflow.
2. MA then fills out Lupron Referral form from Abbvie website. Referral form must be signed by clinician because it includes the prescription.
3. MA creates a Telephone encounter to track the PA.
4. Abbvie reviews the form and obtains a PA from patients insurance. Abbvie will then forward the prescription to the appropriate specialty pharmacy for the patient. They then send a fax to the clinic informing of the pharmacy.
5. MA calls patient letting them know status of PA, and to expect a call from the pharmacy to approve delivery to WHA office.
6. MA calls pharmacy and orders Lupron to be delivered to WHA office and updates the telephone encounter.
7. After Medication is scheduled to arrive, a staff message should be sent to patient services pool for your site to get scheduled for injection on a date that is 2 days after the expected arrival of medication.
8. After patient comes in for injection, MA sends a post-dated staff message to the appropriate pod pool scheduled for 1 week before patient is due for next injection. This will alert the pod that the medication needs to be ordered then follow steps 6-8 as appropriate.

**FAQ's**

- When Abbvie receives the prescription referral form it should only take 3 -4 business days to hear back from them. Please follow up if you don't hear anything back after this time frame.
- There is a Copay assistance option available through the Abbvie website if the patient needs copay help. This medication can be very expensive.
- If patient declines medication for any reason after it was discussed between patient and provider please document appropriately and send as FYI to the provider so they are aware. A different treatment plan may need to be determined.

Women's Healthcare Associates™

# abbvie

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☐ **Dispense as written/Do not substitute** **Date**

I authorize Pharmacy Solutions and its employees to serve as my agent for the sole purpose of obtaining patient benefit information and the necessary prior authorization forms when dealing with Health Plans and Pharmacy Benefits Managers (PBMs), if the plan or PBM requires such authorization.

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**WHA Test Results Management**

Workflow Contact: Kayla Quinn, Amy Palmer, Gary Burgoine, Marni Carlyle

Teams Involved – Clinicians, MA's, LPN's, RN's

**Clinician Resulting Workflow:**

1. **If MyHealth is active**  – Clinicians should send a Result Note (Myhealth message) directly from their In Box to the patient with her results (**Tipsheet ➔ Notifying Patients via MyHealth in a Result Note – If you don't have a Quick Action created**).
    a) If additional follow up is needed based on result (ie abnormal pap-needs colpo), simultaneously route result note to Pod Pool for follow up to ensure she received her results & schedules follow up care.
    b) **Result Note Quick Actions** are the most efficient way to manage repetitive lab results (normal and abnormal) and should be the first tactic used for managing results, not routing this work to MA's or RN's (**Tipsheet ➔ Creating Result Note Quick Actions**).
2. **If MyHealth is Not Active**  – Clinician to send result note to the appropriate pool (Pod or Advice) for notification.
    a) Results that should be sent to **Site Pod Pool for MA/LPNs to manage** (unless you anticipate patient will have many questions that would require RN knowledge or further RN assessment):
        - Most normal labs/pathology & Imaging results
        - Uncomplicated Abnormal pap smears
        - Abnormal Vaginitis panels showing yeast or BV
        - Abnormal 1hr or 3hr GTTs
        - Abnormal ABC/CBC indicating mild anemia with recommendation for addition of PO Iron in pregnancy
        - Abnormal TSH
        - Positive Urine Cultures
    b) Results that **should be sent to the site Advice Pool for RNs to manage**

- **Any of the above** labs or imaging results, or abnormal results **if you anticipate will require RN level explanation or further RN assessment**
- Fertility labs
- HCGs
- PIH panels, labs drawn for cholestasis or other problems in pregnancy (Normal or Abnormal – RNs can review labs, assess for current sx & re-review precautions/warning signs or carry out plan of care)
- FFNs
- Stable, (reassuring) surveillance CA-125

3. **Clinicians should communicate results directly to patients themselves (via the telephone) if:**
   a) New diagnosis of cancer or serious medical condition.
   b) A first, or elevated (slightly or significantly) CA-125, as this will likely precipitate questions or planning.
   c) Results that are unclear or generate uncertainty, meaning a discussion with shared decision making is likely.
   d) You anticipate the patient will need/want significant counseling/explanation of the results because of patient anxiety or past experience where the patient elicits long explanations of results (in which case, planning a telehealth visit in advance is a good idea –"I'll send you a quick message by MyHealth when the results come back but let's schedule a telehealth visit next week so we have enough time to discuss the results in detail.").
   e) You are not certain that an MA or RN can explain the result with the level of detail, knowledge or nuance needed to satisfy the patient (managing uncertainty).

While clinician efficiency is a goal, diligent and kind and thoughtful care takes precedent and is how we ourselves would wish to experience medical care.

**MA/LPN/RN Resulting Workflow**

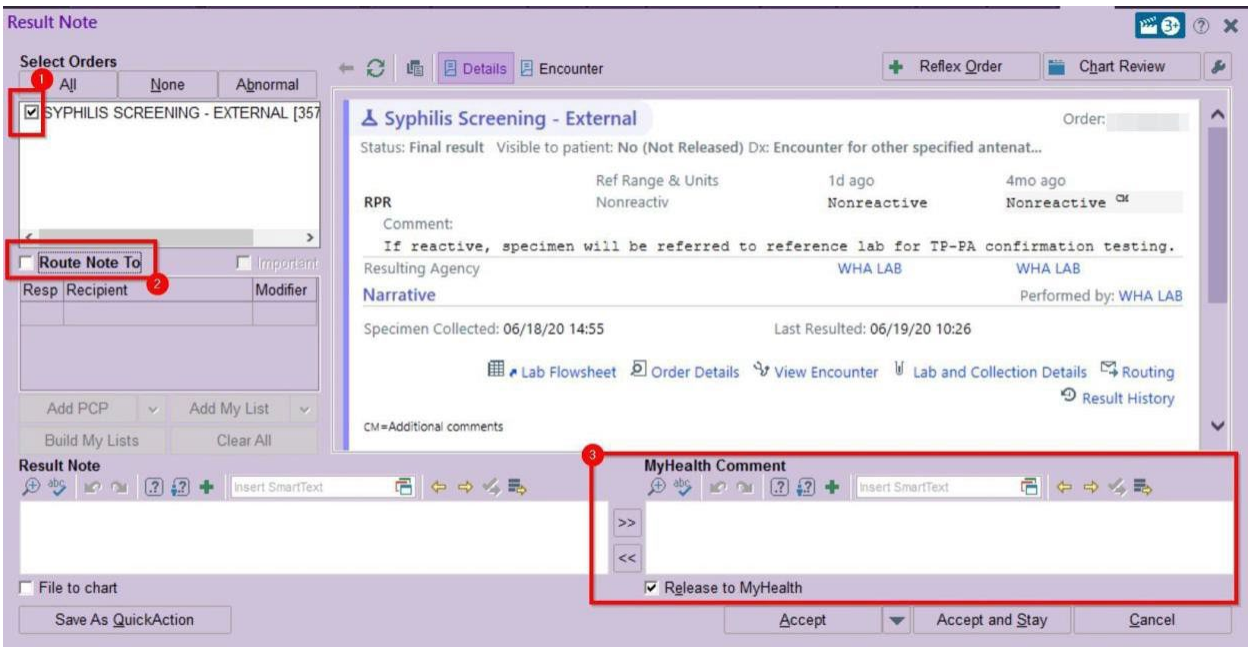
A # of results come directly to the MA/LPNs/RNs without a clinician reviewing them first, as well as results that come to the Pods/Advice pools from the provider for patient notification.

In order to ensure we are not duplicating calls or letters to patients or resulting tests inappropriately to patients the following guidance should be utilized when managing results.

When notifying patients of results from the pool by phone or letter, a result note should be created indicating how the patient is being notified. Ie "Patient notified via phone. See encounter for details." This helps others quickly know the patient was notified of those results when looking at the report & where to find the notification details.

**Normal Results:**

1. MyHealth is active  - All normal results are okay to be sent via MyHealth if active.
   a. **Always verify the result note from the provider against the actual result to ensure they match prior to notification.**
   b. Send results to patient via MyHealth result comment. **When the patient is active on MyHealth it is okay to send those results as they come in vs. waiting for the entire panel to return (ie you don't have to wait for the GC/CT result in order to notify her of her normal OB panel).**
      i. If there is a panel of labs, select the labs that you would like your result Note/MyHealth comment attached to.
      ii. Ensure "Route Note To" is unchecked.
      iii. Type in your result note comment to the patient, "Hi Jane, your recent syphilis screening came back negative. The rest of your results are pending, we will notify you when these are received. Please let us know if you have any questions."
      ****Use caution when releasing results to MyHealth for OB ultrasounds or NIPTs. Make sure patient is aware that the fetal sex will be present on the report in the event they did not want to know that info****




2. MyHealth is **NOT** active 

    a. MA/LPN/RN should review results tab in chart review for any additional labs that may be pending or resulted prior to initiating patient notification, so that result calls or letters can be clustered together when possible.
        i. **Look in chart review for open telephone encounter, letter, or result notes prior to starting a new encounter or letter**
        ii. **Always verify the result note from the provider against the actual result to ensure they match prior to notification.**
        iii. When documenting notification, each test patient was notified of, should be included in your phone documentation, ie, instead of documenting, "patient notified of normal Labs," we should instead document, "Patient notified of negative RPR, HIV, GC/CT, HebB, & Rubella immune, and normal ABC."
    b. MA/LPNs – when resulting tests out to patients, if a patient has a question that you do not have the answer to, these patients should always be sent to an advice nurse to further assist or answer the patient's questions/concerns.

**Normal Results:**

c. The Type of lab needs to be taken into account prior to determining how these need to be resulted:
   i. Normal OB Labs :
      - OB panel with STI screen & Urine Culture (Letter – use .WHAOBPANELRESULTSNOTIFY smart phrase)
      - Pap Smear w/wo HPV testing (Letter)
      - Group B Strep (Telephone Call)
      - GTTs (Telephone Call)
      - Genetic Testing (Telephone Call)
   ii. GYN screening Labs where okay to send normal results via Letter:
      - DEXA
      - Pap Smears w/wo HPV
      - Lipids
      - ABC/CBCs
      - CMPs
      - TSH
      - Other Hormone Testing
   iii. Labs originating from a Problem visit or sensitive labs: Most often should be called if no MyHealth as patients are usually eager for these results. Examples include:
      - ABCs/CBCs
      - TSHs
      - Vaginitis Panels
      - GYN STI tests
      - Cultures
      - UA (micro)
      - Genetic Testing
      - CA 125

**Abnormal Results:**

1. When sent to an MA/LPN/RN from a provider to notify a patient, a telephone call should be the primary method of contacting the patient by the support staff.
    a. Make 3 attempts and document each attempt in a telephone encounter.
    b. If unable to reach may send a letter & attempt notification via MyHealth if active.
    c. Send encounter to the provider as an FYI to keep them in the loop.
2. When the following abnormal labs are encountered by the MA/LPN/RN, need to ensure the problem list has been updated:
    a. **RH negative status on OB Panel** *(ICD10: O26.899 , "Rh negative, maternal")*
    b. **GBS + status from 36wk RV swab** *(ICD 10 O99.820, "GBS (group B Streptococcus carrier) + RV culture, currently pregnant")*
    c. **Abnormal 1hr GTT** *(ICD 10: O99.810; "Abnormal Glucose Tolerance Test in Pregnancy").*
    d. **Abnormal 3hr GTT – Change above to-** *(ICD-10: O24.419; "Gestational diabetes")*
3. Abnormal Cervical Cytology (Paps, colpos,LEEPs) - provider has notified a patient of her abnormal pap via myhealth and sends a simultaneous result note to the pod pool
    a. If patient has not viewed her myhealth message within 7 days of it being sent, MA/LPN to call patient to notify her of her results and transfer to scheduling. Ensure pap tracker was completed.
    b. If patient viewed her myhealth message, ensure pap tracker was completed and send staff message to patient services to call patient to schedule follow up.

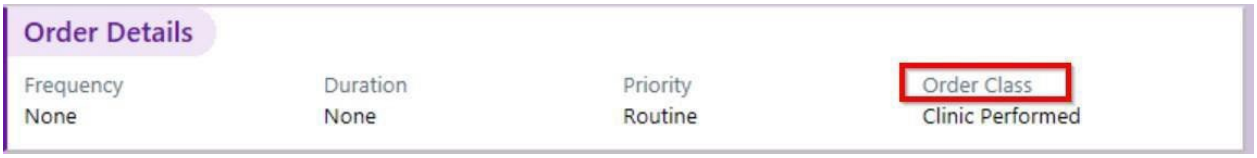
**Overdue Results Management**

Workflow Contact: Kayla Quinn, Amy Palmer

Teams Involved – MA / LPN's, Clinicians

**Order Types: Diagnostic Breast Imaging, Screening Mammogram, Diagnostic Mammogram, Pelvic Ultrasound.**

When looking at imaging orders within the overdue results folder in the pool, first review the order details to determine the order class. This will guide how you proceed:




**Clinic Performed (anticipate completion at WHA)**

1. Look to see if patient is scheduled for a future visit or has already been seen for this service. Ensure order is not a duplicate.
    A. If duplicate order, cancel order.
    B. If patient is currently scheduled to be seen postpone the order to 1wk after patient is scheduled. Once service is completed & resulted the overdue result order will disappear from the pool.
    C. If patient was seen within the last week for this service **at** WHA and no results are in the chart yet – send an email to the helpdesk as the results may be stuck somewhere.
    D. If patient is not scheduled or seen recently, look for a telephone encounter or certified letter from the PS team. Typically they would have already attempted to reach patient to schedule via monitoring of the scheduling orders workqueue.
        i. If no documentation or certified letter, contact patient x3 by phone (May use MyHealth as an additional outreach attempt if active).
            a. If Unable to reach pt, send certified letter, cancel order from overdue results pool & route encounter to

ordering provider as FYI. If the clinician would like any additional follow up action completed, to document this in encounter and route back to pod pool for follow up.

b. If patient contacted she should be encouraged to schedule and transferred to a receptionist to schedule & the order postponed for future follow up.

**Ancillary performed (Anticipate completion at Legacy Facility)**

1. Look to see if patient is scheduled for a future visit or has already been seen for this service. Ensure order is not a duplicate.
    A. If duplicate order, cancel order.
    B. If patient is currently scheduled to be seen postpone the order to 1wk after patient is scheduled. Once service is completed & resulted the overdue result order will disappear from the pool.
    C. If patient was seen within the last week for this service and no results are in the chart yet – reach out to Legacy Facility where service was performed for status on result.
    D. If patient is not scheduled or seen recently, create a telephone encounter and attempt to reach patient x3 by phone (May use myhealth as an additional outreach attempt if active).
        i. If Unable to reach pt, send certified letter, cancel order from overdue results pool & route encounter to ordering provider as FYI. If the clinician would like any additional follow up action completed, to document this in encounter and route back to pod pool for follow up.
        ii. If patient contacted she should be encouraged to schedule and given the Legacy # to schedule.

**Externally Performed (anticipate completion at any other Non-WHA & non-legacy facility - If showing up in your overdue results pool though, it was ordered incorrectly.)**

1. Update Care Everywhere & review the media tab to see if imaging results have returned – Care everywhere results & Media results do NOT satisfy an external imaging order.
    - **a.** If the results have returned and the patient has not been notified. Start a telephone encounter, and route to the ordering provider for review with the following smart phrase pulled in to your note: **.WHAIMAGINGCENOTIFICATION**
        - **i.** Mark that imaging order as "done" from the overdue results pool.
    - **b.** If there are no results available in Care Everywhere or the Media tab of chart review:
        - **i.** If you are uncertain that patient still needs to have imaging done, start telephone encounter, and reach out to provider for clarification of need. Clinician will route encounter back to the pod pool with recommendations.
        - **ii.** If pt still needs imaging, MA/LPN will attempt to reach patient x3 by phone May use myhealth as an additional outreach attempt if active).
            a) If Unable to reach pt, send certified letter, cancel order from overdue results pool & route encounter to ordering provider as FYI. If the clinician would like any additional follow up action completed, to document this in encounter and route back to pod pool for follow up.
        - iii. If patient contacted she should be encouraged to schedule:
            a) MA/LPN to verify where patient intends to have imaging performed, will cancel order from overdue results pool, & re-order correctly as an external imaging referral order, and resend to performing facility (referral navigators will do the future follow up for the external imaging orders when ordered correctly).

**Order Types: Lab Work**

1. Review the chart to determine if the labs are in process. If the order is still "active" and the patient was not seen recently or scheduled in the future for her lab work (this can be found by looking at appointment desk):
    a. Update Care Everywhere to determine if labs were performed at an outside facility
    b. If not, create a telephone encounter. If uncertain if ordered labs are still needed, consult ordering provider for guidance. Clinician will route encounter back to the pod pool (not individual) with recommendations. Otherwise MA/LPN should make 3 attempts to contact patient by phone.
    c. If patient is scheduled or recently seen, postpone the order x1wk after the date she is scheduled. **If the orders were not ordered as standing, and are >30 days old, cancel & re-enter orders so they transfer back into orchard**
    d. If unable to reach patient, route telephone encounter to the ordering provider to determine if a certified letter is indicated. Clinician to route encounter back to pod pool (not individual), advising if certified letter is needed or if okay to cancel order & outreach attempts.
        i. Cancel order

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# Vaginal GC and CT Workflow

Workflow Contact: Kristina Wood

Teams Involved – MA / LPN's, Clinicians, PSR's

Due to a manufacturing shortage of Cervical GC/CT swabs we are currently only offering Vaginal GC/CT swabs. The vaginal collection swabs are intended for patients to self-collect.

**Workflow:**

If a patient calls to get in for STI testing offer them an in person visit or GYN telehealth same day or next day.

- If there is no in-person availability please schedule the patient for a GYN Tele Visit to do STI counseling.
- The provider doing the counseling will determine if GC/CT testing is indicated and if yes: will enter instructions in for the patient to have a lab appointment for STI labs.
- Reception team will call the patient to get the patient scheduled for a lab appointment for self-collection (vaginal) GC/CT and any other tests the provider has ordered. It is recommended that the lab appointment is same day or next day.
- If a Patient is being recalled for treatment of cure GC/CT 3 months post prior positive:
    - If patient has questions regarding this test please schedule a GYN televisit with a provider.
    - If patient has no questions and wants GC/CT swab please schedule a lab visit for the patient to come in and complete a self-collect (vaginal) GC/CT swab.

**Clinical Services:**

Please ensure you are following proper lab labeling guidelines and putting your initials and collection time on the specimen before transporting from the exam room to the lab for processing.

**Clinicians:**

- All GC/CT swabs are vaginal collection swabs. It is ideal for patient to collect this swab. If you find yourself collecting a GC/CT swab, please only collect Vaginal sample NOT Cervical as this requires a different swab.

**Breast Concerns**

Workflow Contact: Amy Palmer, Kayla Quinn

Teams Involved – Clinicians, Advice RN's, PSR's

Workflow for patients calling with breast concerns or at an imaging center with breast concerns

- Patient calls in with breast concern and gets scheduled or sent to the advice RN
- If reaching the advice queue, the nurse schedules a same day or next day appointment for in-person evaluation.
- If same day or next day availability is super limited or patient requests imaging prior to appointment, nurse consults with provider.
    - Provider can choose to order imaging first based on patient availability or anxiety.
- MD decides on any necessary imaging and follow up.
- If patient is at a diagnostic imaging center and a new breast concern is reported by the patient, the nurse talking to DI will have a verbal consult with the clinician.
    - The clinician will choose what is most appropriate for the patient and ask the RN to assist in any scheduling or order placement

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# Fetal Echocardiogram Imaging Order Tip Sheet

Workflow Contact: Kristina Wood

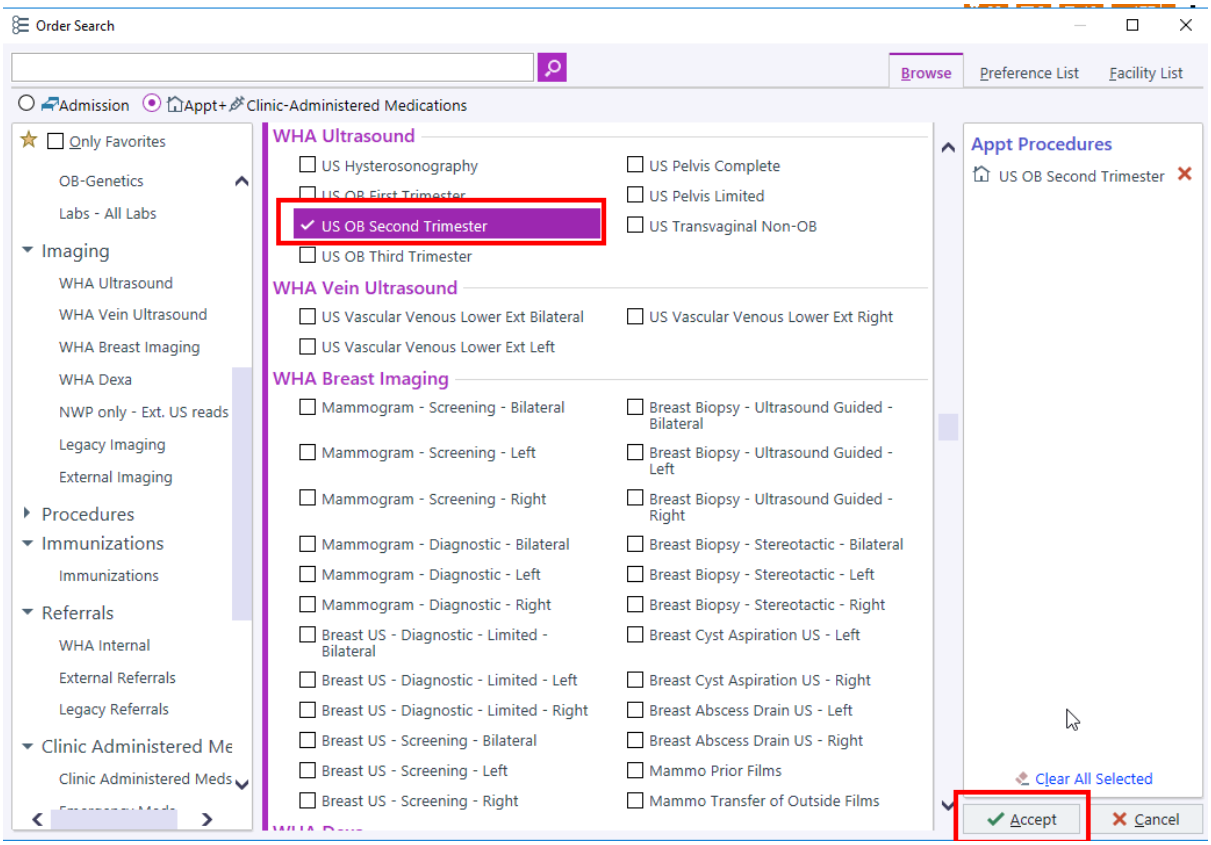
Teams Involved – Clinicians, PSR's

**Background:** Clinicians entered a fetal echocardiogram order AND a NWP referral in order to get a patient scheduled for a fetal echocardiogram.

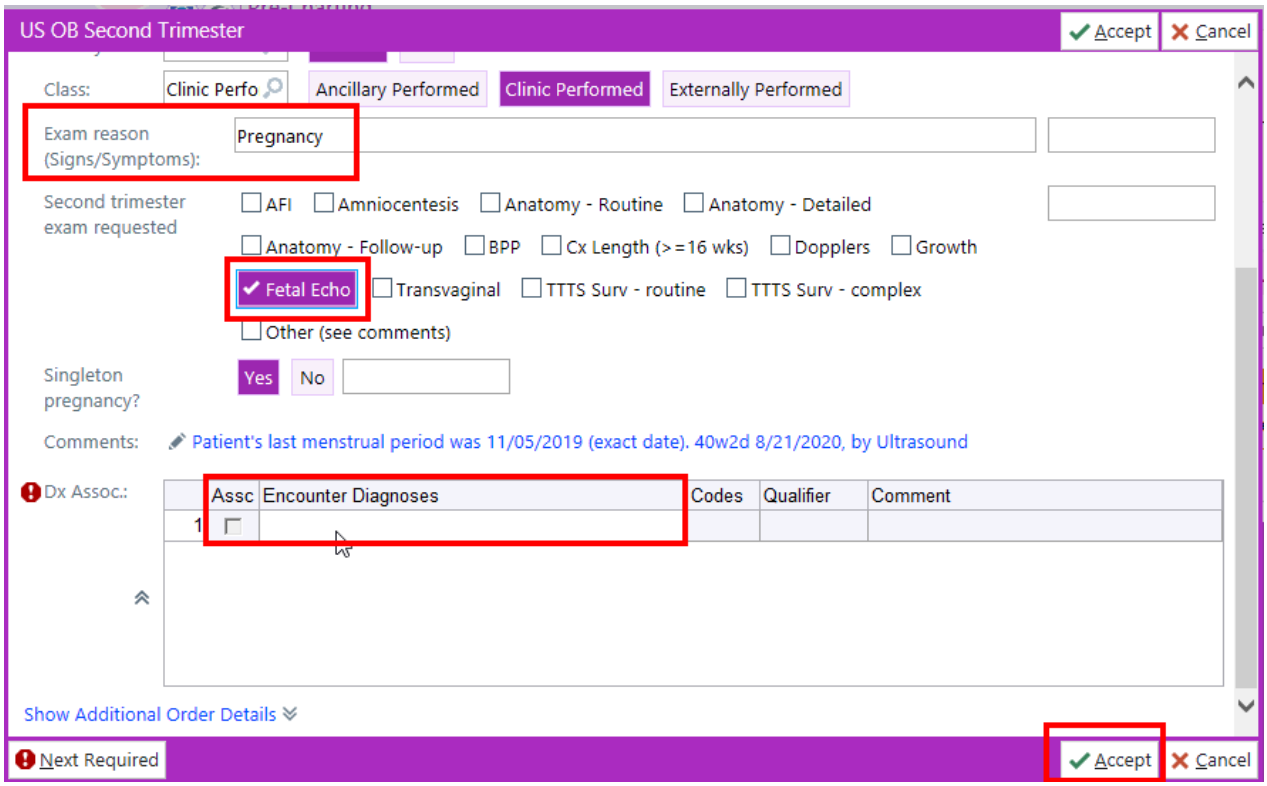
**Change:** Clinicians will no longer need to order the Epic Referral to NWP with the Fetal Echo Imaging Order.

Provider will place the Fetal Echo Imaging order only moving forward. Please see images below.

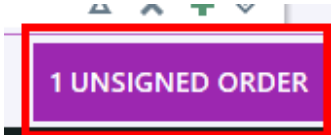
1. Place the order for a second trimester ultrasound




2. Open the OB Second Trimester ultrasound order, fill out exam reason, checkmark the "Fetal Echo", associate your diagnosis and accept your order.




3. Sign your order & you are done. Patient services team will take it from there and your patient will be scheduled appropriately.



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**FMLA/Short Term Disability and Behavioral Health**

Workflow Contact: Caitlin Mackenzie, Katie Snow

Teams Involved – Patient Services, Clinical Team, Behavioral Health, Advice RNs

1. FMLA/STD request to PS Team
2. ROI/Fees- Making sure the MH box is checked/initialed by the patient
3. Medical assistant does initial research and any patient outreach to determine nature of request (amount of time off, etc)
4. Medical assistant determines if goes to medical provider or BH
5. Medical assistant determines claim should be reviewed by BH
6. Medical assistant calls patient to determine desire for leave time/intermittent time
7. Medical assistant inputs dotphrase ".*whabhleaverequestma*" and F2's through prompts
8. Medical assistant signs their documentation and then routes phone encounter to BH pool for pts home site
9. Behaviorist reviews MA's documentation, then inputs their dot phrase ".whabhleaverequest" into the same phone note
10. Behaviorist F2s through the dot phrase to complete documentation
11. Behaviorist routes phone note back to pod pool per the instructions in the MA dot phrase.
12. MA will fill out the physical forms per documentation
13. MA will give forms back to front office for faxing/patient notification

14. If leave request is being denied/unsupported, Behaviorist will reach out to patient via phone to inform and explain.

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## MA portion of the dot phrase



# Behavioral Health Leave Request Initiation

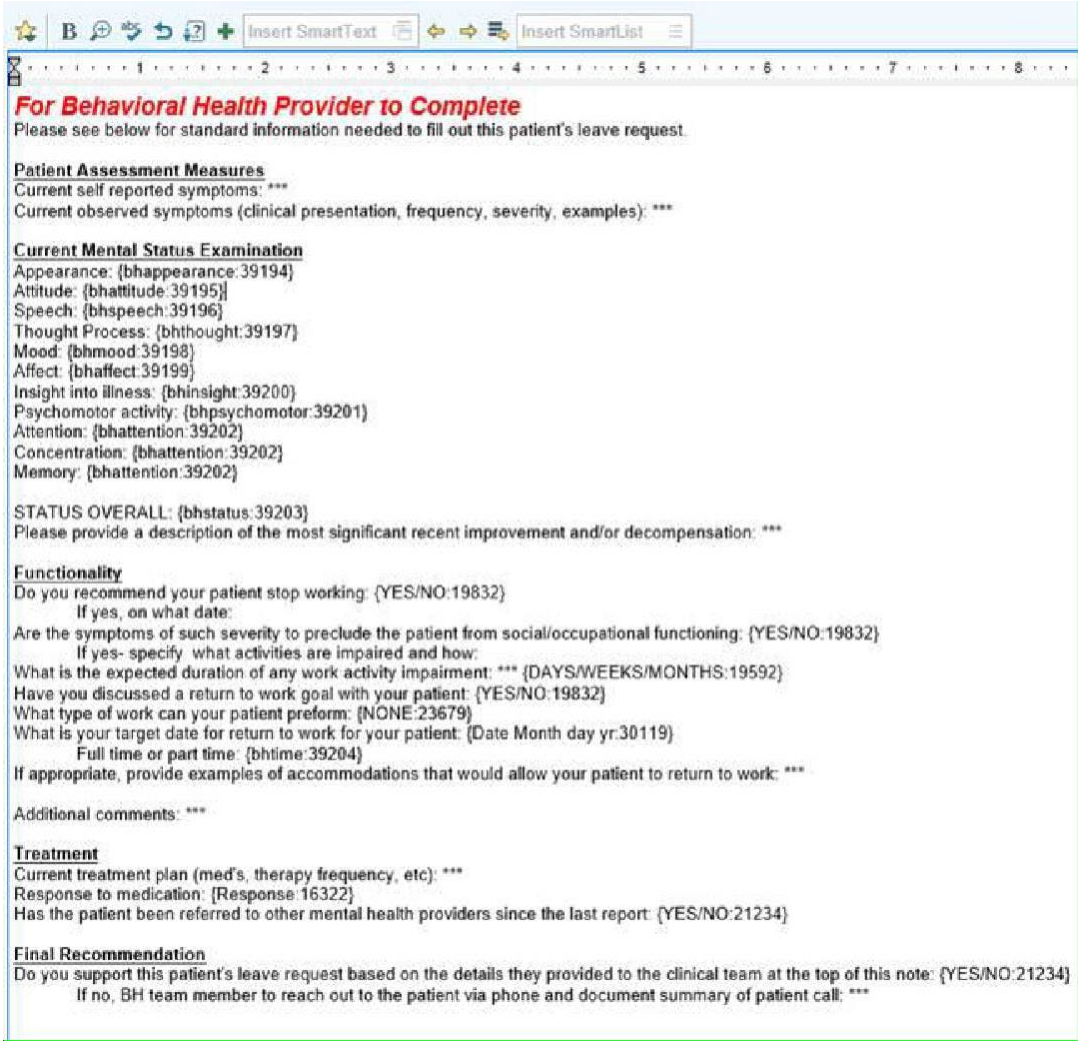
Mouse has requested a leave of absence from work related to her mental health condition.
Current mental health diagnoses in her problem list are: {whabhdx:39192:s}.
Her last behavioral health appointment was on *** with {whabhproviders:39193}.
Mouse is requesting {NUMBER 1-10:19601} {DAYS/WEEKS/MONTHS:19592} off based on a phone conversation with the patient.
Has the patient started her leave already: {YES/NO:21234}
If yes, on what date: {Date Month day yr:19168}
Has the patient brought up this leave request at any recent visit: {YES/NO:21234}

**Instructions for Behaviorist:**

1. Enter *.whabhleaverequest* into the documentation section of this phone note
2. Once completed by behaviorist team, please route back to *** pod pool

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## Behavioral Health portion of dot phrase



***For Behavioral Health Provider to Complete***

Please see below for standard information needed to fill out this patient's leave request.

**Patient Assessment Measures**

Current self reported symptoms: ***
Current observed symptoms (clinical presentation, frequency, severity, examples): ***

**Current Mental Status Examination**

Appearance: {bhappearance:39194}
Attitude: {bhattitude:39195}
Speech: {bhspeech:39196}
Thought Process: {bhthought:39197}
Mood: {bhmood:39198}
Affect: {bhaffect:39199}
Insight into illness: {bhinsight:39200}
Psychomotor activity: {bhpsychomotor:39201}
Attention: {bhattention:39202}
Concentration: {bhattention:39202}
Memory: {bhattention:39202}

STATUS OVERALL: {bhstatus:39203}
Please provide a description of the most significant recent improvement and/or decompensation: ***

**Functionality**

Do you recommend your patient stop working: {YES/NO:19832}
If yes, on what date:
Are the symptoms of such severity to preclude the patient from social/occupational functioning: {YES/NO:19832}
If yes- specify what activities are impaired and how:
What is the expected duration of any work activity impairment: *** {DAYS/WEEKS/MONTHS:19592}
Have you discussed a return to work goal with your patient: {YES/NO:19832}
What type of work can your patient preform: {NONE:23679}
What is your target date for return to work for your patient: {Date Month day yr:30119}
Full time or part time: {bhtime:39204}
If appropriate, provide examples of accommodations that would allow your patient to return to work: ***

Additional comments: ***

**Treatment**

Current treatment plan (med's, therapy frequency, etc): ***
Response to medication: {Response:16322}
Has the patient been referred to other mental health providers since the last report: {YES/NO:21234}

**Final Recommendation**

Do you support this patient's leave request based on the details they provided to the clinical team at the top of this note: {YES/NO:21234}
If no, BH team member to reach out to the patient via phone and document summary of patient call: ***

## Standard turnaround time: 5-7 business days Signature—BHCs sign for one another when needed

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# OB First Trimester Workflow

Workflow Contact: Lindsay Farley

Teams Involved – Patient Services, Clinical team, FC's, Clinicians

## Prenatal Care at WHA 6 – 14 Weeks

### Patient Services Team:

**Medical Receptionists**
Patient calls to let us know she is pregnant: If she indicates that she wants to schedule prenatal care proceed with this workflow, if patient indicates that she is unsure of next steps offer scheduling routine prenatal care, or options counseling.

*Scripting: Would you like to schedule routine prenatal care?* Pause…*or a visit to discuss your pregnancy options?*
If options counseling desired see options counseling workflow

**Prenatal Care Scheduling**
Create telephone encounter enter answers to questions
Review current care model Telehealth vs in clinic person visits and Zero guest policy
Schedule patient for 3 visits below

1. Intake – schedule on the HV INITIAL PRENATAL resources at 6-7 weeks gestation
2. 8 week telehealth- Ideally should be with same person that's is doing the work up and 2nd options is provider with in preferred providers call group (Sub group)
    a. If patient is declining this visit please utilize the following scripting, "In order to proceed with your 12 week ultrasound and in person provider visit, we have to ensure that you complete this appointment. This appointment is necessary for your provider to evaluate your pregnancy and ensure we have the correct ultrasound order in place for that 12 week ultrasound and in person provider visit."
3. Dating and Viability ultrasound, Work up 11.3-13.6 weeks, and Lab draw (ideally prior to 13.4 to allow time for repeat NT if needed)

Complete and route Telephone encounter to OB coordinator pool

Enter care team provider and sub group in patient registration

*Patients who call with unknown LMP or 10-14 week gestational age:*

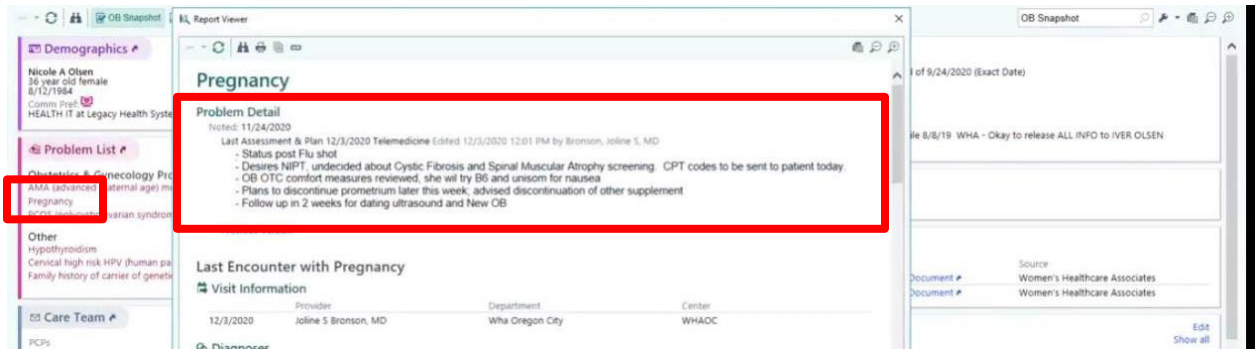
- Medical Receptionist will ensure patient is scheduled for their PI visit within 1 week
- Medical Receptionist will ensure the patient is scheduled within 1 week of their PI visit for their Telehealth OB initial provider visit
- Patient will be scheduled for their OB work up and 40min Ultrasound prior to the 13w6d mark

*Patients who call at 14+ week gestational age:*

- Medical Receptionist will ensure patient is scheduled for their PI visit within 1 week
- Medical Receptionist will ensure the patient is scheduled within 1 week of their PI visit for their Telehealth OB initial provider visit
- Patient to be scheduled for 60min dating and viability ultrasound within 1 week of their Telehealth OB initial provider visit **60min is to accommodate a potential early anatomy scan
    - Please note, if patient is 17-19 weeks gestational age we should schedule the patient at the 20-21 week gestational age mark, this will decrease the need for a repeat ultrasound

**OB Coordinator**

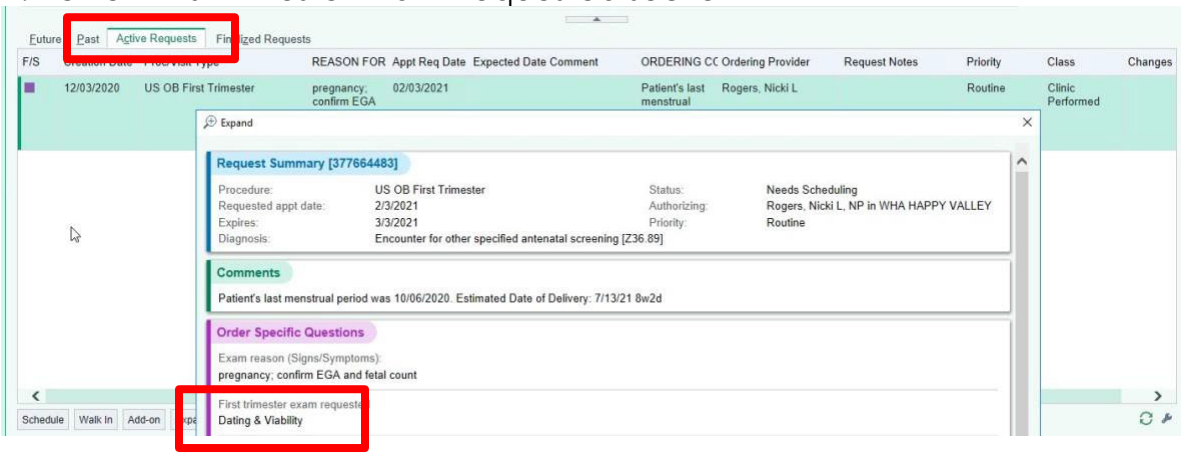
1. Reviews telephone encounter and scheduled visits to ensure accurate scheduling
2. Route staff message to Financial Counselor Pool for benefits. OB Coordinator will document in the telephone encounter that a staff message was sent for benefits check.
3. Postpone Telephone encounter to day after intake to ensure intake completed
    a. if intake not completed follow up with patient and get her rescheduled as needed
    b. if intake completed ensure 8 week and 12 week visits are still scheduled
4. Postpone Telephone encounter to after 8 week visit
    a. Review assessment & plan in the pregnancy problem in the chart to check ultrasound order
        i. In chart on snap shot click Pregnancy
        ii. In pop up screen review top section problem details




1. Desire NIPT or Declines Genetic testing– indicates ultrasound order should be for dating and Viability only
2. SEQ or Sequential testing desired – indicates SEQ Ultrasound D&V with NT

b. Go to patient's appointment desk and look at the active order tab to ensure order matches the Assessment

i. Click on Active Request Tab
ii. Double click on the order
iii. Review First trimester Exam Requested section




c. If order matches link order to ultrasound visit

d. If order does not match text in Pregnancy problem

i. Send a staff message to clinician that performed 8 week visit to enter correct ultrasound order.
ii. Postpone telephone encounter for 3 business days and check back to ensure correct order entered and link to visit.
iii. If the order has not been corrected or entered after 3 business days, resend to Clinician and copy the PCS/PS for the site, note in the staff message 2nd request, postpone 3 business days again and follow up and link order.

5. Post pone to after the 12 week visit to ensure patient had visit or reschedule if needed
6. Once 12 week visit is complete sign encounter
    - If the patient miscarries during the first trimester, OB Coordinator will send a staff message to WHA Billing & Coding Pool indicating end of pregnancy. OB Coordinator will document in the telephone encounter that a staff message was sent to the WHA Billing & Coding Pool then sign off the telephone encounter

**Financial Coordinator Team**

1. Receives staff message indicating patient is scheduled for NOB visits
2. Verifies benefits and contacts patient with insurance coverage
3. Financial counselor enters deposit on patient's account for collection at the appropriate gestational age.

**Clinical Team**

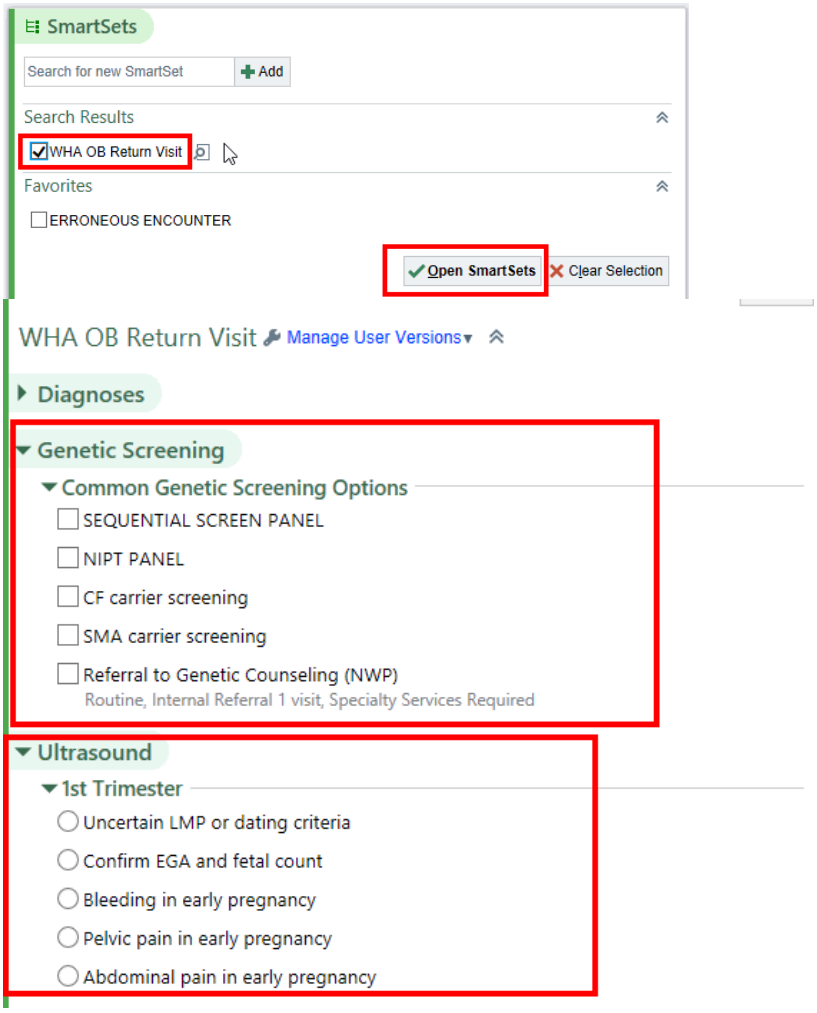
Do not enter 1st trimester ultrasound or genetic testing order: See PI Work flow

**Clinician**

8 week telehealth visit:

1. Review medical history with patient and address any outstanding medical complication
    a. Update problem list
    b. Use smart set to complete all orders and referrals for this visit
2. Review and confirm Genetic testing options
3. Review sites call structure and hospital coverage
4. In plan insert "WHA OB return visit" smart set (see screen shots below)
    a. Click u/s order
    b. Click appropriate genetic orders if indicated
    c. Add any additional appropriate orders/referrals as needed
    d. Sign smart set (this will also sign your orders that are enter in the smart set)

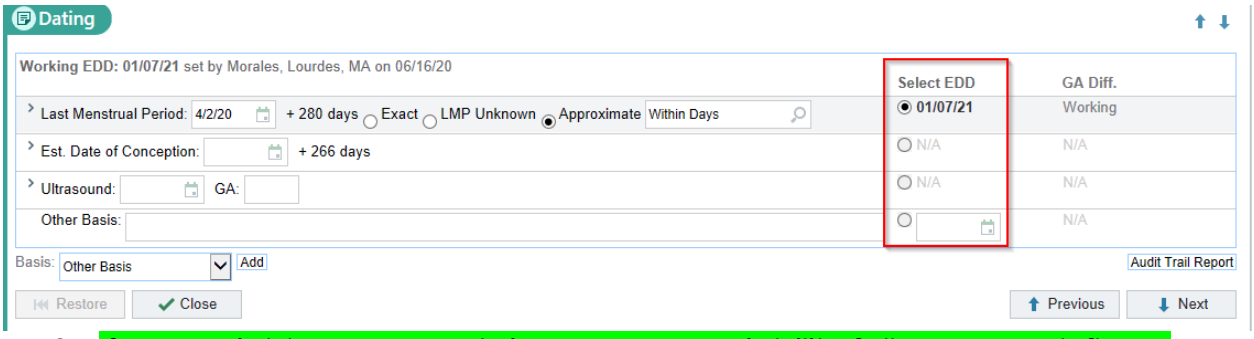
Patient should already be schedule for 12 week visit – OB coordinator is monitoring appointments 0-14 weeks
Smart set use:




12 week in person visit

Ultrasound is completed, provider will
Review ultrasound results with patient

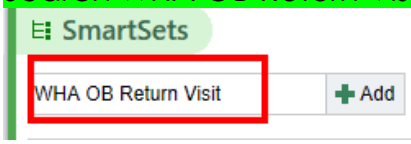
1. If viable pregnancy will move forward with
    i. Pap
    ii. CG/GT if indicated
    iii. Order future labs
    iv. Order 20 week anatomy scan
    v. Discuss on 16 week visit being telehealth
    vi. In wrap up tab Enter **check out note** to schedule 16 week telehealth visit and 20 week anatomy scan and in person visit
    vii. Ensure the appropriate EDD is inputted in the dating section



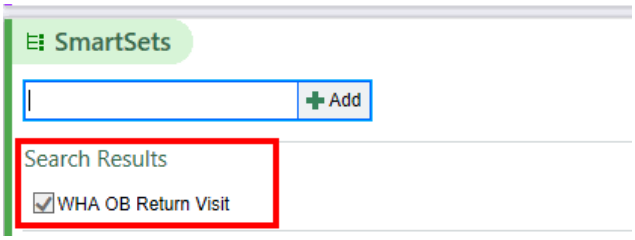

2. If None viable see Uncertain Pregnancy Viability follow up work flows: https://whallc.sharepoint.com/COVID19%20Workflows/Clinical%20Workflows.pdf

## How to add the smart set to your favorites

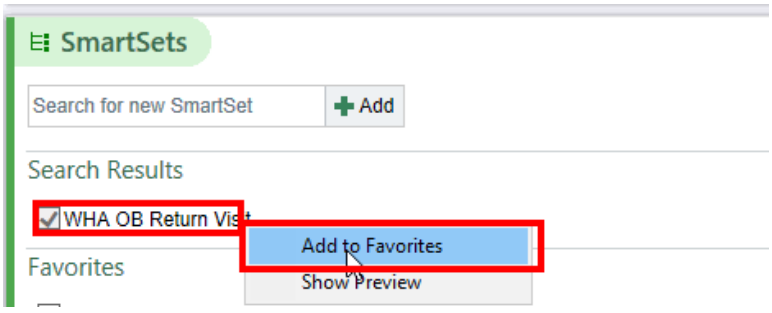
Search WHA OB Return Visit click "Add" or hit "enter"




It will add to "search results"




Right click on the WHA OB Return Visit under 'Search Results' and choose "Add to favorites"



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# Workflow for Patients with Questions about COVID-19 Vaccine

Workflow Contact: Kayla Quinn

Teams Involved – Patient Services, Clinical team, Clinicians

## COVID Vaccine Questions That Come Up During Office Visits

**A. During Office Visits (Pro-active Communication):**

a. Can Review COVID-19 Talking Points (Below)

b. COVID-19 Vaccine FAQs can be inserted into AVS for OB & GYN patients - **.WHACOVID19VACCINEFAQ**

- If your patient has questions that cannot be addressed by the Talking Points & FAQs, send a staff message to the P WHA COVID SCHEDULING TRIAGE NWG-PK and the COVID RNs will follow up with the patient.

*Making your FAQs an AVS speed button*



a. **Clinical/Lab Staff:** Prior to administering any vaccines, clinical staff will verify with patient that she has not or is not scheduled to receive a COVID-19 vaccine within the last or upcoming 14 days.
    i. If a patient says, "YES," vaccine will be held with instruction to discuss further with their clinician during office visit.
    ii. If patient says, "NO," proceed with ordered vaccine as planned.
b. **Clinician Counseling -TDAP:** Patients Who are due for a TDAP and are planning for or recently received a COVID-19 Vaccine:
    i. Clinician will provide counseling on recommendation for 14 day interval between covid-19 Vaccine series & other vaccines and recommendation to delay given that administration of Tdap (if needed) in order to receive intended COVID-19 vaccine.
    ii. If following shared decision making discussion, patient decides to delay her tdap vaccination Clinician can pull in the following smart phrase into their visit note documentation: **.WHATDAPANDCOVIDVACCINECOUNSELING**

*@FNAME@ is due to receive her Tdap vaccine today. She had/plans to have the COVID-19 vaccination on ***. I discussed the CDC recommendation for a minimal interval of 14 days before or after administration of the COVID-19 vaccine and other vaccinations, due to lack of data on the safety and efficacy of the COVID-19 vaccines when administered simultaneously with other vaccines. For this reason, the Tdap vaccination is not being provided today @FNAME@ is counseled that the Tdap vaccine is given in pregnancy to maximize the maternal antibody response and passive antibody transfer to the fetus, with the goal of reducing the risk for neonatal Pertussis. American College of Obstetricians and Gynecologists recommends that the Tdap vaccination be provided between 27 and 36 weeks, thus delaying this vaccination nearer to 36 weeks is within recommended prenatal care. @FNAME@ is advised that it is recommended for all household members to be current on their Tdap vaccination to provide additional protection to the newborn. @FNAME@ opts to postpone their Tdap vaccine due to upcoming or recent COVID-19 vaccination, and will contact our office for an injection visit at least 14 days after dose 2 of the COVID-19 vaccine, which can be completed in conjunction with a routine OB check.*

    iii. Patient should be instructed to schedule their tdap vaccine in conjunction with their future OB check 14 days following their 2nd covid-19 Vaccine.

c. **Clinical Staff:** When prepping charts & rooming patients after 28wks, ensure thorough review of immunizations and need for tdap to ensure this is not missed.

**Incoming Calls or Emails re: COVID-19 Vaccine Safety**

1. **Incoming Calls: Patient Services –**Send telephone encounter to site Advice Pool for return call from nurse.
    a. **Advice RN** will ask patient if she currently has access or is being offered a COVID-19 vaccine
        i. If YES- and pregnant and lactating she will complete the COVID Warm Handoff Smart phrase to gather additional information and route to the COVID clinic team to review COVID-19 Vaccine talking points with patient (below).
        ii. If NO – Advice RN will discuss talking point re: Pregnant and lactating people have been excluded in the recent vaccine trials; therefore, there are no data on the safety of the COVID-19 vaccines in pregnant people. Further studies are ongoing, and safety data will become available in the coming months. This will help guide decision-making for people who are not yet included in this phase of vaccination. If the vaccine becomes available to you, reach back out to us at that time, and we will be able to provide the most up to date information.

2. **Patient Emails Questions about Safety of COVID-19 Vaccines in pregnancy or Lactation**
    a. Advice RN will pull in COVID-19 Vaccine FAQs (.WHACOVID19VACCINEFAQ) into Myhealth message and send to patient and encourage her to let us know if further questions.
    b. If further questions – Warm hand off will be completed, and routed to COVID Clinic Team.

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**Incoming Calls or Emails about Scheduling a COVID-19 Vaccine**

1. **Incoming Calls: Patient Services or Nursing Team–** Will Communicate the following to the patient:
    - "The Oregon Health Authority (OHA) is prioritizing eligibility status and managing the distribution of vaccine. WHA is not currently able to offer COVID-19 vaccines to patients. At this time, we are not keeping a waitlist for future vaccination. If this changes, we will let you know. For more information on determining your eligibility and vaccination sites near you, please visit the OHA's COVID-19 vaccine website, which is updated regularly: https://covidvaccine.oregon.gov/. This website may direct you to Legacy MyHealth to sign up for a COVID-19 vaccine. WHA is not affiliated with Legacy's vaccine events. We understand this process is cumbersome, but unfortunately, we are not able to influence or assist you. If you're not able to find an appointment, you will need to check back at a later time for more available openings."

2. **Incoming Emails re: Scheduling COVID-19 Vaccine:**
    - Advice RN will input the following smart phrase (.WHAADVICECOVIDVACCINESCHEDULINGQUESTIONS) with information on how to sign up for COVID-19 Vaccines into a MyHealth Message and send back to patient.

**WHA ADVICE PROTOCOL: Gonorrhea & Chlamydia Co-Infection**

Workflow Contact: Kayla Quinn, Amy Palmer

Teams Involved – Advice RNs, Clinicians (as FYI Only)

**Smart Phrase for Gonorrhea & Chlamydia Co-infection (GYN or OB):**

.WHAADVICEPOSCOINFECTIONGONOCHLAM

*Positive result for Gonorrhea TPA* ***AND*** *Chlamydia co-infection. Prior to contacting the patient her chart was reviewed and allergies clarified. (Smart link to allergies).*
*Current weight: {smart link}*
*LMP: ****
*Pregnancy Status: {pregnant, not pregnant, unknown} – If Pregnancy Status is unknown, UPT will be completed at time of Rocephin injection. If Positive, Rx for Azithromycin will be sent instead of Doxycycline.*
*No drug allergy contraindications to standard treatment.*

- ***Non-Pregnant:*** *Patient is less than 300lbs and will require treatment with a single dose of 500mg of Rocephin IM* ***PLUS*** Doxycycline 100mg BID PO x 7 days
- ***Non-Pregnant:*** *Greater than 300lbs and will require treatment with a single dose of 1g of Rocephin IM* ***PLUS*** doxycycline 100mg BID PO x 7 days

- ***Pregnant:*** *Patient is less than 300lbs and will require treatment with a single dose of 500mg of Rocephin IM* ***PLUS*** Single dose of Azithromycin 1gm PO
- ***Pregnant:*** *Greater than 300lbs and will require treatment with a single dose of 1g of Rocephin IM* ***PLUS*** Single dose of Azithromycin 1gm PO

*Known allergy, and patient reports previous symptoms of severe cutaneous reaction or difficulty breathing following cephalosporin use.*

- Single dose of 240mg Gentamicin IM **PLUS** a single dose of 2g azithromycin.
  **Gentamicin & Azithromycin order sent to pharmacy. Patient to pick up Gentamicin IM medication from pharmacy and bring to clinic for IM administration.*

*Known allergy to azithromycin, erythromycin, other macrolides,*

*cephalosporins or penicillins and patient reports previous Sx of severe cutaneous reaction, or difficulty breathing following use:*

- Local health department will be contacted for recommended alternative treatment options.

*Patient notified of positive gonorrhea result and*

- *An immediate nurse visit was scheduled for *** & PO medications sent to pharmacy*

*OR*

- *Reviewed alternative recommended therapy with patient due to allergies and to bring prescribed medication to her appointment for administration*

- *Reviewed due to multiple drug allergies to standard treatments, we would contact the health department for alternative treatment options and call her back to review.*

*Complete STD screening was reviewed and offered; she:*

- ***Accepts** : Orders entered for - HIV; RPR; Hepatitis B surface antigen; Hepatitis C virus antibody*
- ***Declines***

*Patient was notified that she will need re-screening at 3 months. Orders placed.*
*She was informed that WHA is required to report her infection to the Oregon Health Authority and that her local health department may contact her to ask a few questions and ensure both she and her partner(s) are treated.*
*Patient instructed to refer partners with whom she has had sexual contact in the past 60 days for evaluation, testing, and presumptive treatment as best option to receive first line therapy.*
*Patient feels confident that her partner [will/ will not] get tested and treated with PCP or County Health department as recommended.*
*Patient Declines / Accepts Expedited Partner Therapy.*
*Reviewed importance of completing the PO antibiotic course in its entirety, and not missing doses to ensure clearance of infection. Discussed option for Zofran, to take prior to taking the doxycycline or to take with food for better tolerance. Reviewed side effects of Zofran. Patient (accepts/declines) this Rx.*
*She was advised to avoid sex for 7 days after both she and her partner(s) have completed the antibiotics to avoid re-infection.*

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**Manual actions that need to be completed by RN (Below is for Information only, delete from note prior to routing to clinician):**

- Enter Problem of Gonorrhea to problem list (if not already present) and copy assessment to CPOE
- Order appropriate additional STI tests if desired by patient
- Schedule nurse visit for IM injection and lab visit for lab draws if desired.
- Enter orders for appropriate therapy:

| **Gonorrhea-Chlamydia Co-Infection Treatment** | | **Allergy to first line therapy - OB or GYN** |
|---|---|---|
| **GYN**<br>(Clinic Administered Med Order, Plus pharmacy Rx)<br><br>If < 300lbs treat with one time dose of 500mg Rocephin IM injection<br>**Or**<br>If >300lbs treat with one time dose of 1g Rocephin IM injection<br><br>**PLUS**<br>doxycycline 100mg BID x 7 days<br><br>**May offer:**<br>Zofran 4mg BID #14 x 0<br>To increase adherence to regimen | **OB**<br>(Clinic Administered Med Order, Plus pharmacy Rx)<br><br>If < 300lbs treat with one time dose of 500mg Rocephin IM injection<br>**Or**<br>If >300lbs treat with one time dose of 1g Rocephin IM injection<br><br>**PLUS**<br>Single dose of Azithromycin 1gm PO | (Rxs for both meds sent to pharmacy for pick up - Plus Clinically administered med order for gentamicin)<br>One time dose of 240 mg gentamicin IM<br>**PLUS**<br>single 2 g oral dose of azithromycin is an option |

- Enter post-dated order for 3mo GC/CT rescreen (do not use test of cure order).
- Enter 3mo recall for f/u testing or transfer patient to schedule 3mo f/u test
- Complete OR Health Department Infectious Disease form and send to local health department using web based reporting form: https://epiweb.oha.state.or.us/fmi/webd/Memento%20Morbi?homeurl=http://healthoregon.org/howtoreport
  **OR** print and fax from link below: http://www.oregon.gov/oha/PH/DiseasesConditions/CommunicableDisease/ReportingCommunicableDisease/ReportingForms/Documents/morbrpt.pdf
- Route note to provider so they are notified of protocol completion.

**WHA ADVICE PROTOCOL: Expedited Partner Therapy – Chlamydia & Gonorrhea**

Workflow Contact: Kayla Quinn, Amy Palmer

Teams Involved – Advice RNs, Clinical Support Staff, Clinicians (as FYI Only)

**Smart Phrase for Expedited Partner Therapy**
.WHAADVICEEXPEDITEDPARTNERTHERAPY
*She was informed that it would be best to have all partners thought to have been exposed during the previous 60 days go to a clinic for examination, testing and treatment with first line therapy. She was asked if her partner(s) had a primary doctor or would seek treatment in a timely fashion. She was not certain her partner(s) would get examination and treatment so* ***EPT was initiated*** *per CDC, ACOG and State of Oregon recommendations and guidelines.*

- *Partner's name: ****
- *Allergies:*
  - *NKDA*
  - *Known drug allergy to [azithromycin, erythromycin, other macrolides, cephalosporins or penicillins]*
  - *Known Allergy to other Drug: ****
- *Current medications: ****
- *Treatment:*
  - *No allergy contraindications to EPT:*
    - ***Chlamydia Only****:: Azithromycin 1gm PO Once*
    - ***Gonorrhea Only****: Cefixime (Suprax*) 800 mg (8 x 100 mg / 4x 200 mg / 2 x 400 mg) orally*
    - ***Gonorrhea & Chlamydia Co-infection****: Cefixime (Suprax*) 800 mg (8 x 100 mg / 4x 200 mg / 2 x 400 mg) orally* ***plus*** *Doxycycline* 100mg BID x7 days *orally.*
  - *Known allergy to component of EPT, RN will refer to local county health department for partner treatment and # will be given to patient*
- *Pharmacy used: ****
- *Additional Partners (complete above information for each additional partner): [Yes / No]*

*Printed information was sent to the patient to give to her partner(s) including contact information for her partner to use to report an adverse EPT event to WHA or for them to contact WHA for further information. She was informed that her partner should seek care urgently in the event of an adverse reaction.*

**Actions to be completed by RN:**

- Enter this smart phrase to same Telephone encounter as notification note.
- Send EPT Rx's to preferred pharmacy if no allergy contraindications.
- Send supplemental information to patient to give to her partner(s) if EPT initiated: Gonorrhea Partner information - Verify with patient how they would like this sent to them.
- Send supplemental information to patient to give to her partner(s) if EPT initiated: Chlamydia Partner Information - Verify with patient how they would like this sent to them.
- Route note to provider for awareness of completion of protocol.

Women's Healthcare Associates LLC

**WHA ADVICE PROTOCOL: Gonorrhea**

Workflow Contact: Kayla Quinn, Amy Palmer

Teams Involved – Advice RNs, Clinical Support Staff, Clinicians (as FYI Only)

**Smart Phrase for Positive Gonorrhea (GYN or OB):**

.WHAADVICEPOSGONORRHEAONLY

*Positive result for Gonorrhea TPA. She does not have a co-infection with Chlamydia. Prior to contacting the patient her chart was reviewed and allergies clarified.*
*(Smart link to allergies).*
*Current weight: (smart link)*
*No drug allergy contraindications to standard treatment (OB or GYN):*

- *Patient is less than 300lbs and will require treatment with a single dose of 500mg of Rocephin IM*
- *Greater than 300lbs and will require treatment with a single dose of 1g of Rocephin IM*

*Known allergy, and patient reports previous symptoms of severe cutaneous reaction, or difficulty breathing following cephalosporin use.*
*Recommended Treatment:*

- *Single dose of 240mg Gentamicin IM **Plus** a single dose of 2g azithromycin.*
  **Gentamicin & Azithromycin order sent to pharmacy. Patient to pick up gentamicin IM medication from pharmacy and bring to clinic for IM administration.*

*Known allergy to azithromycin, erythromycin, other macrolides, cephalosporins or penicillins and patient reports previous Sx of severe cutaneous reaction or difficulty breathing following use:*

- *Local Health Department will be contacted for alternative treatment recommendations*

*Patient notified of positive gonorrhea result and*

- *an immediate nurse visit was scheduled for ****

  *OR*

- *Reviewed alternative recommended therapy with patient due to allergies and to bring prescribed medication to her appointment for administration*

*OR*

- *Reviewed due to multiple drug allergies to standard treatments, the health department would be contacted for alternative options and she would receive a call back.*
-

*Complete STD screening was reviewed and offered and she:*

- ***Accepts** : Orders entered for - HIV; RPR; Hepatitis B surface antigen; Hepatitis C virus antibody*
- ***Declines***

*Patient was notified that she will need re-screening at 3 months. Orders placed.*
*She was informed that WHA is required to report her infection to the Oregon Health Authority and that her local health department may contact her to ask a few questions and ensure both she and her partner(s) are treated.*
*Patient instructed to refer partners with whom she has had sexual contact in the past 60 days for evaluation, testing, and presumptive treatment as best option to receive first line therapy.*
*Patient feels confident that her partner (will/ will not) get tested and treated with PCP or County Health department as recommended.*
*Patient (Declines / Accepts –opens additional protocol) Expedited Partner Therapy.*
*She was advised to avoid sex for 7 days after both she and her partner(s) have completed the antibiotics to avoid re-infection.*

**Manual actions that need to be completed by RN (Below is for Information only, delete from note prior to routing to clinician):**

- Enter Problem of Gonorrhea to problem list (if not already present) and copy assessment to CPOE
- Order appropriate additional STI tests if desired by patient
- Schedule nurse visit for IM injection and lab visit for lab draws if desired.
- Enter orders for appropriate therapy:

| Gonorrhea Alone<br>OB or GYN - No allergies | | Allergy to first line therapy - OB or GYN |
|---|---|---|
| (Clinic Administered Med Order)<br><br>If < 300lbs treat with one time dose of 500mg Rocephin IM injection | (Clinic Administered Med Order)<br><br>If >300lbs treat with one time dose of 1g Rocephin IM injection | (Rxs for both meds sent to pharmacy for pick up - Plus Clinically administered med order for gentamicin)<br>One time dose of 240 mg gentamicin IM **PLUS** single 2 g oral dose of azithromycin is an option |

- Enter post-dated order for 3mo GC/CT rescreen.
- Enter 3mo recall for f/u testing or transfer patient to schedule 3mo f/u test
- Complete OR Health Department Infectious Disease form and send to local health department using web based reporting form: https://epiweb.oha.state.or.us/fmi/webd/Memento%20Morbi?homeurl=http://healthoregon.org/howtoreport
  **OR** print and fax from link below: http://www.oregon.gov/oha/PH/DiseasesConditions/CommunicableDisease/ReportingCommunicableDisease/ReportingForms/Documents/morbrpt.pdf
- Route note to provider so they are notified of protocol completion.

Women's Healthcare Associates LLC

**Nurse Consults and Response Times**

Workflow Contact: Amy Palmer, Kayla Quinn

Teams Involved – Advice Nurses, Clinicians

**Consult Best Practices:**

- New or Worsening issues should be scheduled as a telehealth or in-person visit whenever possible, instead of defaulting to a consultation with the clinician.
- If you need a same day answer, and you haven't heard from the clinician, don't rely on routed epic notes as your sole means of communicating with the provider. Reach out via phone, text (or page if they are on-call & this is listed as their preference on the call scheduler) as appropriate to ensure able to follow through on care with patient in timely manner.
- Post Ops & Complicated GYN/Patients with Chronic Pelvic Pain should be managed by their responsible clinician/Surgeon whenever possible, instead of an on-call clinician or other clinician unfamiliar with their care. (Ie Post Op Pain medications should be addressed by surgeon, Pain contract patients addressed by responsible provider ect).
- Only utilize High priority Flag- if a truly urgent/same day need.
- **Note Documentation**:
    - Always include at the beginning of your note, "patient of **X** calling about" – this will help you identify the responsible provider that the note should be routed to.
    - At the end of your note, document, "Patient aware Dr. X will be out of the office until X date, and understands that it may take a few days from the time the clinician returns to address this question. She is okay with this." --- This is used when the provider is truly out of the office on day off or on vacation.
- **Contacting clinicians that you need same day/urgent answers from (*Unless STAT urgent need, give providers time to respond in epic first so they are not inundated with texts during the day):***
    - Always identify yourself with each text or IM – This is Kayla, RN from WHA.
    - Do not try to have long text consults or problem solve via text or IM, instead your message should say (after introducing yourself), "I

routed an epic message to your desktop to review. Needs to be addressed today. You can call me at # if prefer verbal consult."

**Who To consult for each site in varying scenarios:**

| Site / scenario | Hospital or L&D Consults/FYIs | Urgent Consults Needing Same Day Answer | Non Urgent Consults |
|---|---|---|---|
| **TN** | PFTC | PFTC – even if PFTC is not the responsible provider | Route to responsible provider, providing anticipatory guidance re: timing of response from clinician based on guidelines below |
| **NWG – TN** | n/a | Use Responsible Provider if Available (in-Office); If Not available utilize one of their NWG-TN partners (Maryam Hadiashar, Jennifer Lin, Gina Allison, Carrie Dennison) | Route to responsible provider, providing anticipatory guidance re: timing of response from clinician based on guidelines below |
| **OC** | MD on Call for MD patients CNM on-Call for CNM patients | Use responsible provider if available (in-office, telehealth, on-call)<br>If responsible provider is not in office reach out to MD on-call for MD patients and CNM on-call for CNM patients. | Route to responsible provider, providing anticipatory guidance re: timing of response from clinician based on guidelines below |
| **NB** | PFTC | PFTC – even if PFTC is not the responsible provider | Route to responsible provider, providing anticipatory guidance re: timing of response from clinician based on guidelines below |

| | | | |
|---|---|---|---|
| **PKN/PKS/HB** | PFTC / On-Call CNM | Use responsible provider if available (in-office, telehealth, on-call)<br>If Responsible provider is not in office reach out to COVID clinic provider | Route to responsible provider, providing anticipatory guidance re: timing of response from clinician based on guidelines below |
| **NWG-PK** | n/a | Use responsible provider if available (in office, telehealth, on-call)<br>If Not available utilize one of their NWG-TN partners first (Beth Skrypzak, MD, Lee Mukul, MD, and Chi Chi Abanulo, MD)<br>*Always CC responsible clinician/Surgeon for awareness of issues if consulting other provider* | Route to responsible provider, providing anticipatory guidance re: timing of response from clinician based on guidelines below |
| **BR** | **On-Call Clinician** | Use responsible provider if available (in-office, telehealth, on-call)<br>If Responsible provider is out of the office, utilize On-call clinician (If POGA is primary call- utilize BR back up call provider)<br>*CC responsible clinician for awareness*<br>May use COVID clinic provider if On-call is very busy/unable to answer as needed | Route to responsible provider, providing anticipatory guidance re: timing of response from clinician based on guidelines below |
| **LD** | PFTC | **Use responsible provider if available** (in-office, telehealth, on-call)<br>If responsible provider is not available, use on-call clinician | Route to responsible provider, providing anticipatory guidance re: timing of response from clinician based on guidelines below |
| **GW/HV** | MD on Call for MD patients CNM on- | Use responsible provider if available (in-office, telehealth, on-call)<br>If responsible provider is not in | Route to responsible provider, providing anticipatory guidance re: timing of response from |

| | | | |
|---|---|---|---|
| | ~~Call for~~ CNM patients | ~~office reach out to MD on-call for~~ MD patients and CNM on-call for CNM patients. | ~~clinician based on~~ guidelines below |
| **MBC** | CNM On-Call | CNM On-Call | CNM In-Clinic |

**Triage Criteria:**

- **The Following patient care issues should be addressed within the same business day, either with an in-person visit/L&D visit, telehealth visit (if available) or consultation with clinician:**
    - Patient's Concern or issue could lead to severe injury, illness or death if not addressed same day
        - Ie post op concerns, ectopic pregnancy, hemorrhaging, pre-ecclampsia sx ect.
    - Patient is upset about her care, we made an error or disappointed the patient with our lack of follow up.

- **Non-urgent patient care questions or issues that would be best addressed by patient's primary clinician should be addressed within 2-3 business days. Non-urgent is determined through shared decision making with patient and clinical assessment by RN, and should be documented in the note.**
    - For providers who are out of the office for a prolonged period of time, can anticipate a response within **2-3** business days of their return.